

THE CORE FIX

A Woman's Guide to Healing Diastasis
Recti, Rebuilding Your Core, and
Getting Strong After Baby

Hannah Wells

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For every woman who has been told it's all in her head
—
your body was telling the truth.
This book is for learning to listen,
and for finding your way back to yourself.

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Foreword

I REMEMBER THE FIRST TIME I TRIED TO DO A SIT-UP AFTER HAVING MY BABY.

I had been cleared for exercise at my six-week checkup. My doctor had given me the standard postpartum instructions: start slowly, listen to your body, do your Kegels. So I lay down on my yoga mat, put my hands behind my head, and tried to sit up. Instead of my abs engaging the way they used to, my stomach coned upward — a visible ridge running down the center of my abdomen like a tent pole. It looked wrong, and it felt worse.

I had diastasis recti — the separation of the rectus abdominis muscles along the midline connective tissue (the linea alba). It affects up to 60 percent of women after pregnancy. And almost nobody talks about it.

The standard advice — do your Kegels, wait for your body to bounce back, start doing crunches again when you feel ready — is not just unhelpful for diastasis recti. It can actively make it worse. Traditional ab exercises like crunches, sit-ups, and planks put pressure on the separated connective tissue and widen the gap. The very exercises that are supposed to strengthen your core can actually deepen the separation.

Diastasis recti is not just a cosmetic issue. Yes, it often looks like a pooch or a bulge in the lower belly. But it can also cause lower back pain, pelvic floor dysfunction (including incontinence and prolapse), digestive issues, and a

core that simply does not function the way it should. Healing it is not about getting your pre-baby body back. It is about restoring the structural integrity of your core so you can lift your child, carry groceries, run, jump, and move through life without pain or dysfunction.

The good news: diastasis recti is treatable — and in most cases, it is treatable without surgery. The right exercises (deep core engagement, transverse abdominis activation, proper breathing mechanics, and careful loading) can close the gap and restore core function. The wrong exercises can widen it. The difference between the two is knowledge.

This book is the resource I wish someone had handed me on that yoga mat. It covers what diastasis recti actually is, how to check yourself for separation at home, the healing timeline (because it takes months, not weeks), the breath and posture foundations that everything else rests on, a complete safe core workout, what exercises to avoid, the critical pelvic floor connection, how to move in daily life with a healing core (lifting, carrying, getting out of bed), when surgery is the right option, and how to maintain your progress for life.

Every chapter ends with a toolkit summary you can use immediately.

I am not a doctor, a physical therapist, or a personal trainer. I am a health writer who has healed her own diastasis recti and spent years researching the condition. This book is a guide, not a prescription. Work with a pelvic floor physical therapist for a personalized assessment and program — and if you cannot access one, this book will help you understand what to look for and what to ask for.

Your core is not broken. It is stretched. And what is stretched can heal.

Toolkit Summary

- **Diastasis recti affects up to 60% of postpartum women** — a separation of the abdominal muscles along the midline that can be healed with the right approach.
- **Traditional ab exercises make it worse** — crunches, sit-ups, and planks increase the gap. You need a different approach.
- **Healing takes time** — the connective tissue heals slowly. Expect 3-6 months of consistent work to close the gap.
- **The pelvic floor connection is essential** — you cannot fix your core without your pelvic floor, and vice versa.
- **Surgery is not usually necessary** — most cases can be healed with the right exercise program.
- **I am not a doctor** — this book is a companion, not a prescription. Work with a pelvic floor physical therapist for personalized guidance.

ch01

IF YOU HAVE EVER LOOKED DOWN AT YOUR STOMACH AND SEEN A RIDGE OR BULGE RUNNING DOWN THE CENTER WHEN YOU SAT UP FROM LYING DOWN, YOU HAVE ALREADY SEEN DIASTASIS RECTI FIRSTHAND. YOU JUST MIGHT NOT HAVE KNOWN WHAT TO CALL IT.

Diastasis recti is a separation of the two sides of the rectus abdominis muscle — the "six-pack" muscle — along the connective tissue midline known as the linea alba. The word diastasis comes from the Greek, meaning "to separate." The word recti refers to the rectus abdominis. So diastasis recti literally means the separation of the rectus muscles.

But that clinical definition does not capture what it actually feels like. A separated core means that the deep stabilizing system of your abdomen is no longer working as a unified structure. It means that when you try to engage your abs, the force goes sideways instead of forward. It means your connective tissue has been stretched beyond its capacity, and it needs time and the right type of load to heal.

Understanding the anatomy of this separation is the first step to healing it.

The Muscles Involved: The Rectus Abdominis

The rectus abdominis is a paired muscle that runs vertically from your pubic bone up to your ribcage. If you are in good shape and have low body fat, you can see the divisions between the segments of this muscle — that is the

"six-pack" appearance. But the rectus abdominis is not one single muscle. It is two parallel strips of muscle, separated down the middle by a band of connective tissue.

Each rectus muscle is encased in a sheath of fascia — a thin, strong layer of connective tissue that holds the muscle in place and connects it to the rest of your core system. The outer edges of the rectus muscles connect to the external obliques (the muscles on the sides of your torso that help you twist and bend). The inner edges of each rectus muscle meet at the midline, separated by that band of connective tissue: the linea alba.

When your core is healthy, the two rectus muscles sit close together, side by side, with the linea alba forming a stable bridge between them. When you engage your core, the two muscles contract and the linea alba transmits that force across the midline. Your core works as a single, unified unit.

In diastasis recti, the two rectus muscles have pulled apart. The linea alba has stretched and thinned, creating a gap between the muscles. That gap is the diastasis.

The Connective Tissue: The Linea Alba

The linea alba — Latin for "white line" — is a band of connective tissue that runs from your sternum down to your pubic bone. It is made primarily of collagen fibers that cross and interweave like the threads of a fabric. In a healthy abdomen, the linea alba is about 5 to 10 millimeters wide and has enough tensile strength to handle the forces of coughing, laughing, lifting, and the internal pressure of pregnancy.

The linea alba is not a muscle. It does not contract. It does not generate force on its own. Its job is to transmit force from one side of the rectus abdominis to the other and to hold the abdominal contents where they belong. Think of it as the central tendon of the abdominal wall — the place where all the forces of your core converge.

The linea alba is made of connective tissue, and connective tissue heals differently than muscle. Muscle has a rich blood supply, which is why a pulled muscle can heal in a few weeks. Connective tissue has poor blood supply. When you damage or overstretch connective tissue — whether through pregnancy, surgery, or repetitive strain — it heals slowly and imperfectly. That is why diastasis recti does not resolve on its own for many women and why the right approach to healing takes months, not weeks.

What Separation Actually Means

Diastasis recti is a gap between the left and right rectus abdominis muscles. But the gap itself is only part of the picture. The connective tissue of the linea alba has also been stretched and weakened. Even if the muscle gap is relatively small, if the linea alba is thin and lax, the core cannot function properly.

When you have diastasis recti, the following things happen:

- **Loss of midline tension.** The linea alba can no longer transmit force across the midline efficiently. Instead of force traveling from one side of your core to the other, it pushes out through the weakened midline.
- **Coning or doming.** When you sit up from lying down, or when you cough, laugh, or strain, the abdominal contents push against the weakened midline, creating a visible ridge or bulge. This is called coning (a ridge running vertically) or doming (a bulge localized at one spot).
- **Reduced core stability.** The deep core system — transverse abdominis, pelvic floor, diaphragm, multifidus — relies on the integrity of the connective tissue midline. When the midline is compromised, the entire core system is compromised. This can lead to back pain, pelvic floor dysfunction, and poor movement quality.
- **Compensatory movement patterns.** When your core cannot stabilize you properly, other muscle groups take over. Your lower back tightens. Your hip flexors get overactive. Your shoulders round forward. These compensations create their own problems over time.

Diastasis recti is not dangerous in the sense that it will kill you. But it can significantly affect your quality of life — your ability to lift your child, to run, to exercise, to feel strong in your body.

Causes Beyond Pregnancy

Pregnancy is the most common cause of diastasis recti — up to 60 percent of postpartum women have some degree of separation. But pregnancy is not the only cause. Anyone can develop diastasis recti, regardless of whether they have given birth.

Abdominal surgery. Any surgery that cuts through the linea alba — including C-sections, hysterectomies, and hernia repairs — can cause or worsen diastasis recti. A C-section involves cutting through the fascia and muscle layers of the abdominal wall. Even after the incision heals, the structural integrity of the midline may be compromised. Women who have had multiple C-sections are at higher risk.

Heavy lifting. Chronic heavy lifting with poor core mechanics — whether from weightlifting, manual labor, or moving furniture — can gradually stretch the linea alba. The problem is not lifting itself but lifting without proper intra-abdominal pressure management. When you hold your breath and bear down during a heavy lift, the pressure pushes out against the midline. Over years, this can create a separation.

Obesity. Excess abdominal weight creates chronic intra-abdominal pressure that pushes against the linea alba. Over time, this pressure can stretch the connective tissue and cause the rectus muscles to separate. Weight loss alone does not always reverse the separation because the connective tissue has been permanently stretched.

Age and connective tissue disorders. Aging naturally reduces collagen production, which can weaken the linea alba. Certain connective tissue disorders — including Ehlers-Danlos syndrome and other collagen-related conditions — make the linea alba inherently more stretchable, increasing the risk of diastasis recti even without other risk factors.

Multiple pregnancies. Each pregnancy stretches the linea alba further. Women who have had multiple pregnancies — especially close together — are at higher risk of developing diastasis recti that does not resolve on its own.

If you have diastasis recti, you are not alone. The condition is extremely common. And in most cases, it is treatable without surgery.

The Different Types of Diastasis Recti

Diastasis recti is not one-size-fits-all. The separation can occur in different locations, with different widths, and with different tissue qualities. The type of diastasis you have affects what exercises will be most beneficial.

Above the belly button (supra-umbilical). This separation is located above the navel. It is most common in women who carried high during pregnancy or who did a lot of crunches or sit-ups postpartum. The gap may be narrow or wide, but the tissue above the belly button tends to be firmer than the tissue below.

Below the belly button (infra-umbilical). This separation is located below the navel. It is more common in women who carried low, who have had C-sections, or who

have a history of heavy lifting. The tissue below the belly button tends to be thinner and more stretchable because the lower abdomen has less structural support than the upper abdomen. This type of diastasis is often harder to close and takes longer to heal.

Wide versus narrow gap. A narrow gap (less than two finger-widths) is generally easier to close because the connective tissue has not been stretched as far. A wide gap (three finger-widths or more) takes longer to heal and may require more consistent, long-term work. A very wide gap (four or more finger-widths) may not close completely with exercise alone and may eventually require surgical repair — though many women still see significant improvement with conservative treatment.

Full-length separation. Some women have separation from the sternum all the way to the pubic bone. This is the most challenging type of diastasis because the entire linea alba has been affected. Full-length separation usually requires a longer healing timeline and careful attention to avoiding midline pressure in all activities.

The tissue quality factor. Beyond the width and location of the gap, the tone of the connective tissue matters immensely. Some women have a gap that is only two finger-widths wide but the tissue in between feels thin and squishy, like overstretched elastic. Other women have a wider gap but the tissue feels firmer and more resilient. Tissue quality — how much collagen integrity remains — is a strong predictor of how quickly healing will happen.

Why Connective Tissue Heals Slowly

The connective tissue of the linea alba is made of collagen fibers arranged in a crisscross pattern. This structure gives it strength in multiple directions. When you stretch the tissues beyond what they can handle — during pregnancy, for example — some of those collagen fibers tear. Others just stretch and lose their tension.

The body can repair this damage, but it does so slowly. Connective tissue has a limited blood supply, and blood carries the nutrients and cells needed for repair. While a muscle strain might heal in four to six weeks, a connective tissue injury can take months or even years to fully remodel.

The healing process for connective tissue happens in three overlapping stages:

- 1. INFLAMMATION (FIRST 6 WEEKS).** THE BODY SENDS INFLAMMATORY CELLS TO THE AREA TO CLEAR OUT DAMAGED TISSUE AND BEGIN THE REPAIR PROCESS. DURING THIS PHASE, THE TISSUE IS FRAGILE AND EASILY RE-INJURED. THIS IS WHY THE FIRST SIX WEEKS AFTER PREGNANCY — OR AFTER ANY INJURY TO THE ABDOMINAL WALL — ARE CRITICAL.
- 2. PROLIFERATION (6 WEEKS TO 6 MONTHS).** NEW COLLAGEN FIBERS ARE LAID DOWN TO FILL THE GAP. HOWEVER, THE NEW COLLAGEN IS DISORGANIZED AND NOT VERY STRONG. THIS IS THE PHASE WHERE THE RIGHT KIND OF MECHANICAL LOAD — GENTLE CORE WORK, PROPER BREATHING — CAN HELP GUIDE THE COLLAGEN FIBERS TO ALIGN IN THE RIGHT DIRECTION.

3. REMODELING (6 MONTHS TO 2 YEARS). THE DISORGANIZED COLLAGEN IS GRADUALLY REPLACED BY ORGANIZED COLLAGEN THAT IS STRONGER AND MORE ALIGNED WITH THE DIRECTION OF FORCE. THIS IS THE PHASE WHERE THE TISSUE TRULY REGAINS ITS STRENGTH. FULL REMODELING CAN TAKE UP TO TWO YEARS.

The implication is clear: healing diastasis recti is a marathon, not a sprint. Consistent, patient work over many months produces the best results.

The Range of Severity

Diastasis recti exists on a spectrum. On one end, a woman might have a fingertip-width gap that she only notices when she actively checks for it. On the other end, a woman might have a palm-width gap with visibly thin tissue, significant coning, and associated symptoms like back pain and pelvic floor dysfunction.

The severity of diastasis recti is typically classified by:

- **Width of the gap.** How many finger-widths can you fit between the rectus muscles at the widest point? Less than two finger-widths is generally considered mild. Two to three finger-widths is moderate. More than three finger-widths is severe.
- **Depth of the gap.** When you palpate the gap (feel it with your fingers), how deep is it? A shallow gap (just the surface tissue) is less concerning than a deep gap that extends through multiple layers of the abdominal wall.

- **Tissue quality.** Does the tissue at the midline feel firm and springy, or thin and squishy? Tissue quality is a better predictor of healing potential than width alone.
- **Tension.** Can you feel tension in the midline when you engage your core, or does the tissue feel slack? Some tension is a good sign — it means the connective tissue still has some structural integrity.
- **Associated symptoms.** Diastasis recti that is causing back pain, pelvic organ prolapse, urinary incontinence, or digestive issues is more severe than diastasis that is purely cosmetic.

Most women fall somewhere in the mild-to-moderate range. The common image of a dramatic, visible bulge is at the severe end of the spectrum. But even a mild separation that is causing back pain or affecting your exercise quality deserves attention and treatment.

A Note on Diagnosis

While this book teaches you how to assess your own separation at home, a formal diagnosis from a pelvic floor physical therapist, a physical therapist, or your doctor is the gold standard. Research shows that self-assessment can be reasonably accurate for detecting diastasis recti, but a trained professional can evaluate the quality of the connective tissue, assess associated pelvic floor function, and rule out other conditions like umbilical hernias.

If you suspect you have diastasis recti — and especially if you have symptoms like back pain, incontinence, or a visible bulge — schedule an assessment with a pelvic health professional. And if you cannot access one easily, the self-check technique is a reliable starting point.

Toolkit Summary

- **Diastasis recti is a separation of the rectus abdominis muscles** along the connective tissue midline (linea alba), affecting up to 60 percent of postpartum women.
- **The linea alba is connective tissue, not muscle** — it heals slowly because of limited blood supply, and full tissue remodeling takes 6 months to 2 years.
- **Pregnancy is the most common cause, but not the only one** — abdominal surgery, heavy lifting, obesity, age, connective tissue disorders, and multiple pregnancies can all cause or worsen separation.
- **Diastasis recti has several types** — above or below the belly button, wide or narrow gap, full-length separation — and the type affects treatment.
- **Severity is measured by width, depth, tissue quality, tension, and associated symptoms** — from fingertip-width (mild) to palm-width (severe).
- **A formal diagnosis from a pelvic floor PT is ideal** — self-assessment is helpful but not a substitute for professional evaluation.

ch02

IF YOU FEEL LIKE YOUR CORE HAS GONE MISSING SINCE YOU HAD YOUR BABY, YOU ARE NOT IMAGINING IT. YOUR ABDOMINAL MUSCLES DID NOT JUST GET WEAK FROM NINE MONTHS OF DISUSE. THEY WERE PHYSICALLY STRETCHED, SEPARATED, AND HORMONALLY SOFTENED. THE "GAP" BETWEEN YOUR MUSCLES IS NOT A SIGN THAT YOU HAVE LOST YOUR FITNESS. IT IS A STRUCTURAL CHANGE THAT HAPPENED BECAUSE YOUR BODY MADE ROOM FOR A GROWING HUMAN.

Understanding what happened to your core during pregnancy — and what happens in the weeks and months after birth — is essential to healing it. When you know why your abs feel different, you can work with your body instead of against it.

What Happens to Your Core During Pregnancy

Your body did not betray you during pregnancy. It did exactly what it was designed to do. But the changes that allowed your baby to grow also left your core in a different structural state than it was before.

The abdominal muscles stretch. As your uterus expands, it pushes forward against the abdominal wall. The rectus abdominis muscles — the two vertical strips of muscle that run down your front — are attached to the ribcage and

pubic bone, so they cannot move out of the way. Instead, they stretch. The muscle fibers elongate to accommodate the growing uterus. By the third trimester, your rectus muscles may be stretched to nearly double their original length.

The muscles themselves are not damaged in most cases — they are just elongated. After birth, they can shorten back to their original length, provided they are not being held in a stretched position by poor posture or by the weight of a baby carried on the front of your body.

The linea alba thins and widens. The connective tissue midline between your rectus muscles — the linea alba — also stretches during pregnancy. This is not a bad thing. Some separation of the linea alba is normal during pregnancy; it creates space for the uterus to expand without tearing the abdominal wall. The linea alba is designed to stretch, within limits.

The problem is that for some women, the linea alba stretches beyond its capacity to snap back. The collagen fibers are overstretched, and some of them tear. After birth, the linea alba remains wider and thinner than it was before pregnancy. The two rectus muscles are now separated by a wider gap, and the connective tissue that spans that gap is weaker.

Hormones soften everything. Two pregnancy hormones play a major role in diastasis recti: relaxin and progesterone.

Relaxin is produced by the ovaries and placenta during pregnancy. Its job is to relax the ligaments of the pelvis to allow for childbirth. But relaxin does not only affect the pelvis — it affects all the connective tissues in your body,

including the linea alba. Under the influence of relaxin, the collagen fibers of the linea alba become more pliable and stretchable. This is helpful for accommodating the growing uterus but makes the midline more vulnerable to overstretching.

Progesterone also softens connective tissue and affects muscle tone. During pregnancy, progesterone slows down the digestive system (which is why many pregnant women experience constipation) and relaxes smooth muscle throughout the body, including the muscles of the abdominal wall.

These hormonal changes do not reverse overnight after birth. Relaxin levels drop quickly after delivery but can remain elevated for several weeks if you are breastfeeding. Some women have measurable relaxin levels for up to six months postpartum. During this time, your connective tissue remains softer and more stretchable than normal — which means it is easier to overstretch it further with the wrong exercises or movements.

What Happens After Birth: The Recovery Window

In the first days and weeks after birth, your body begins to reverse the changes of pregnancy. The uterus contracts back down to its pre-pregnancy size. The hormones begin to rebalance. The abdominal muscles start to shorten back toward their original length.

For some women, the gap between the rectus muscles closes naturally during this period. Studies suggest that in the first eight weeks postpartum, a significant number of women see their diastasis resolve without any specific intervention. For others, the gap persists.

Why does the gap close naturally for some women and not for others? Several factors seem to matter:

- **The degree of separation during pregnancy.** Women who had a wider separation in the third trimester are more likely to have a persistent gap postpartum.
- **Connective tissue quality.** Women with naturally stretchier collagen (or conditions that affect collagen, like Ehlers-Danlos syndrome) are less likely to see natural closure.
- **Muscle tone before pregnancy.** Women who had good core muscle tone before pregnancy may have muscles that spring back more effectively — but this is not always the case. Even very fit women can develop diastasis recti.
- **Birth factors.** Women who had a longer labor, a larger baby, or a C-section may have more stretching or damage to the abdominal wall.
- **Activity in the early postpartum period.** What you do in the first weeks after birth affects whether the gap closes or remains open.

The first six to eight weeks after birth are the period when the linea alba has the best chance of natural closure. The tissues are still in the early stages of healing. The hormonal

environment is still shifting. Gentle movement and good posture can support natural closure. The wrong movements — especially anything that creates intra-abdominal pressure through the midline — can work against it.

What Prevents Natural Closure

If the gap does not close on its own in the first weeks and months after birth, something is interfering with the healing process. The most common culprits are:

Weak deep core. The deep core muscles — primarily the transverse abdominis (TVA), the pelvic floor, and the multifidus (a deep spinal muscle) — are responsible for stabilizing the torso. When these muscles are weak or not firing properly, they cannot pull the rectus muscles back together. The TVA acts like a corset, wrapping around the torso and drawing the abdominal contents inward. If the TVA is not engaging, there is nothing to counteract the outward pressure that keeps the gap open.

Many women cannot feel their TVA engaging after birth. This is not because the muscle has disappeared. It is because the neuromuscular connection has been disrupted by the stretching and hormonal changes of pregnancy. The brain can no longer easily find the muscle to activate it. Rebuilding this connection is the first step in healing diastasis recti.

Poor breathing mechanics. How you breathe affects how your core works on a fundamental level. If you are a "belly breather" — expanding your belly outward when you inhale — you are creating intra-abdominal pressure that pushes against the weakened midline. Over time, this

pressure keeps the gap from closing. Switching to a ribcage-dominant breath (expanding the ribcage laterally and posteriorly) can dramatically reduce pressure on the midline.

Most people are not aware of their breathing patterns. They have been breathing the same way their whole lives. Changing how you breathe is possible, but it takes practice.

Carrying your baby incorrectly. How you carry your baby throughout the day affects your core. If you carry your baby on one hip, you are loading one side of your core more than the other. This asymmetric loading can pull the rectus muscles further apart on the loaded side and create rotational stress on the linea alba.

Many women also develop a posture called "baby belly" — they let their belly hang forward (a continuation of pregnancy posture) and their lower back arches. This position keeps the rectus muscles in a lengthened, separated state. The body cannot shorten and close the gap if you are constantly holding the muscles in a stretched position.

Returning to exercise too soon with the wrong exercises. This is one of the most common reasons diastasis recti persists. Women are cleared for exercise at their six-week postpartum checkup, and they return to their pre-pregnancy workout routine — or at least what they remember of it. They do crunches, sit-ups, planks, pushups, and burpees. And each of these exercises increases the pressure on the midline and widens the gap.

The exercises that most people think of as "core work" — crunches, sit-ups, ab machines — are exactly the exercises that make diastasis recti worse. They strengthen the

superficial rectus abdominis (the muscle you can see in the mirror) while either ignoring or actively antagonizing the deep core muscles that actually close the gap. This is not a question of working hard enough. It is a question of working on the right muscles in the right way.

Subsequent pregnancies. If you become pregnant again before your diastasis has healed, the second pregnancy will almost certainly stretch the midline further. Each pregnancy compounds the separation. This is why many women do not discover they have diastasis recti until after their second or third baby — they had a small gap after the first that they did not notice, and the second pregnancy pushed it into the symptomatic range.

The Tissues Involved: Connective Tissue, Muscle, and Fascia

Your core is not just one type of tissue. It is a complex system of three types of tissue that heal at different rates and need different types of support.

Connective tissue (the linea alba). This is the slowest-healing tissue in your core. Connective tissue is made of collagen fibers arranged in a dense, organized lattice. It has limited blood supply, which means it cannot deliver repair cells and nutrients to the area quickly. The linea alba takes months to years to fully remodel after being overstretched.

Connective tissue responds best to tension — the right amount of mechanical load applied in the right direction. Too much tension (heavy lifting without proper core engagement)

damages it. Too little tension (not doing any core work at all) does not stimulate repair. The sweet spot is gentle, consistent loading that tells the collagen fibers to lay down in the right orientation.

Connective tissue also needs good nutrition to heal — specifically, adequate protein, vitamin C (which is essential for collagen synthesis), and water.

Muscle (the rectus abdominis and deep core). Muscle heals relatively quickly compared to connective tissue. A mild muscle strain can heal in a week or two. Muscle responds to exercise by getting stronger — this is the principle of progressive overload.

But you cannot simply strengthen the rectus abdominis and expect the gap to close. In fact, strengthening the rectus alone without addressing the deep core can make the separation worse. The superficial abs will pull the rectus muscles into a shorter, tighter position, but they will not close the gap at the midline. You need to strengthen the transverse abdominis — the deep corset muscle — to pull the rectus muscles back together from the inside.

Muscle needs the right type of load (not too much, not too little), adequate protein for repair, and good neuromuscular connections (your brain needs to be able to find the muscle).

Fascia. Fascia is the thin, web-like connective tissue that wraps around every muscle, organ, and bone in your body. It is the body's internal packaging material. In the abdomen, the fascia that encases the rectus muscles (the rectus sheath) is continuous with the fascia of the external obliques, internal obliques, and transverse abdominis.

Fascia is often the overlooked tissue in diastasis recti. The stretching of pregnancy does not just affect the linea alba — it affects the entire fascial web of the abdomen. The fascial connections between the rectus muscles and the obliques can become lax or restricted. This fascial disorganization contributes to the feeling of having a "dysfunctional" core that does not respond the way it used to.

Fascia responds to hydration, gentle movement, and consistent low-level tension. Fascial remodeling takes time — similar to connective tissue — and cannot be rushed.

Why "Just Give It Time" Is Not Enough

You have probably heard the advice to "give your body time" to bounce back after pregnancy. And there is truth to it: the early postpartum period is a time of profound physiological change, and the body does some of its best healing when left alone.

But time alone does not close diastasis recti. The gap does not close by waiting. It closes by doing the right things — and not doing the wrong things.

The linea alba will not spontaneously regenerate to its pre-pregnancy width while you are carrying your baby on one hip, doing crunches at the gym, and breathing into your belly. Those activities actively oppose healing. You need to create the conditions for healing: proper breathing, proper posture, proper core engagement, and avoidance of midline pressure.

The gap closed during pregnancy because your body was doing exactly what it needed to do to make space for your baby. Healing the gap is the same process in reverse — but it requires intentional action, not just patience.

Toolkit Summary

- **Pregnancy causes three changes to your core:**
the rectus muscles stretch lengthwise, the linea alba thins and widens, and hormones (relaxin and progesterone) soften all connective tissue.
- **Natural closure happens for some women in the first 6-8 weeks** — but many factors (degree of separation, tissue quality, birth factors, early activity) determine whether the gap closes on its own.
- **Four things commonly prevent natural closure:**
weak deep core (especially TVA), poor breathing mechanics (belly breathing), carrying baby asymmetrically, and returning to the wrong exercises too soon.
- **Three tissue types are involved:** connective tissue (heals slowly over months to years), muscle (responds to exercise but needs the right type), and fascia (needs hydration and gentle movement). Each needs different support.
- **Time alone does not heal diastasis recti** — you need to create the right conditions for healing through intentional movement, breath, and posture. Talk to a pelvic floor PT for personalized guidance.

ch03

BEFORE YOU CAN HEAL YOUR DIASTASIS, YOU NEED TO KNOW WHAT YOU ARE WORKING WITH. HOW WIDE IS THE GAP? WHERE IS IT LOCATED? DOES THE CONNECTIVE TISSUE FEEL FIRM OR THIN? TRACKING THESE MEASUREMENTS OVER TIME GIVES YOU OBJECTIVE FEEDBACK ON WHETHER YOUR CORE WORK IS WORKING — AND LETS YOU CATCH PROBLEMS EARLY IF YOUR GAP IS WIDENING INSTEAD OF CLOSING.

The self-check technique described in this chapter is a standard assessment used by physical therapists. It is reliable enough for tracking progress at home. However, it is not a substitute for a professional evaluation by a pelvic floor physical therapist, especially if you have symptoms like incontinence, pelvic pain, or pelvic organ prolapse. If you are unsure about your self-check results, or if your gap is wide, see a professional.

The Self-Check Technique

You can check yourself for diastasis recti at home. You do not need any special equipment — just your hands and a flat surface to lie on.

The self-check is done in a supine (lying on your back) position with your knees bent and feet flat on the floor. You lift your head and shoulders slightly off the floor — just enough to engage your rectus abdominis — and feel for the gap with your fingers.

Here is the step-by-step process.

Step 1: Set up your position.

Lie on your back on a firm, flat surface — a yoga mat on the floor is ideal. A soft bed will not give you accurate results because your pelvis will sink into the mattress. Bend your knees and place your feet flat on the floor, hip-width apart. Your arms should be at your sides with your palms facing down.

Take a moment to relax. Let your back be in a comfortable, neutral position — not pressed flat into the floor and not arched. Let your belly soften.

Step 2: Locate the midline.

Place two or three fingers (index, middle, and ring) on the midline of your abdomen, just above your belly button. Your fingers should be pointing toward your feet, with your fingertips at the midline and the pads of your fingers spanning across the gap.

You will feel a small depression or hollow at the midline in most people — this is normal. What you are looking for is the width and depth of that hollow.

Step 3: Lift your head and shoulders.

Gently lift your head and shoulders off the floor — a small curl, not a full sit-up. Your shoulder blades should just barely clear the floor. You only need enough engagement to make the rectus muscles contract and reveal the gap.

As you lift, you should feel the rectus muscles come together on either side of your fingers. If there is a gap, you will feel your fingers sink into the space between the muscles. The wider the gap, the more fingers you can fit between the muscles.

Step 4: Measure the gap at the belly button.

With your head and shoulders lifted, gently probe the midline with your fingers. How many finger-widths can you fit between the rectus muscles?

Use the following scale: - **Less than 1 finger-width:** Normal range. Minimal or no separation. - **1 to 2 finger-widths:** Mild separation. You can likely close this with consistent core work. - **2 to 3 finger-widths:** Moderate separation. This will take longer but is still very treatable with exercise. - **3 to 4 finger-widths:** Moderate to severe separation. This needs consistent, patient work. Consider seeing a pelvic floor PT. - **More than 4 finger-widths:** Severe separation. You should definitely see a pelvic floor PT for an assessment.

Step 5: Measure 2 inches above the belly button.

Move your fingers to a point roughly two inches (about two finger-widths) above your belly button. Repeat the same lift-and-probe technique. Measure the width of the gap and note the depth — how far down can you press before you feel resistance?

The gap above the belly button is often wider than the gap at the belly button. The upper abdomen has different structural properties than the lower abdomen, and the gap above the navel is more likely to be influenced by core exercises.

Step 6: Measure 2 inches below the belly button.

Move your fingers to a point roughly two inches below your belly button. Repeat the check. The gap below the belly button is often narrower but the tissue may feel thinner and less resilient. This is also the area most affected by C-sections.

Note: If you have had a C-section, be gentle around the scar tissue. The scar itself is not part of the diastasis gap, but the tissue surrounding the scar may feel different than the rest of your abdomen.

Step 7: Check the full length.

Run your fingers along the entire midline from your sternum (the bone at the top of your chest) down to your pubic bone, with gentle head lifts every inch or so. Some women have separation along the entire length of the midline. Others have separation only in one section.

Make a mental note of where the gap is widest, where it narrows, and whether there are any "soft spots" where the tissue feels particularly thin.

What to Measure

Your self-check is most useful when you track it systematically. Here is what to measure and record:

Width at three points. Record the number of finger-widths at the belly button, two inches above, and two inches below. Use the same fingers each time for consistency.

Depth. How deep is the gap? Press gently into the gap with your fingers. If you can feel the posterior rectus sheath (the back wall of the rectus sheath), that is the full depth of the separation. Depth is an important indicator because it reflects how many layers of the abdominal wall have been affected.

Tissue quality. This is harder to quantify but just as important. Does the tissue at the midline feel firm and springy, like a good elastic band? Or does it feel thin and squishy, like overstretched elastic that has lost its tension? Firm tissue has a better chance of regaining its strength. Squishy tissue needs more careful management and a longer healing timeline.

Coning or doming. While you are doing the self-check, look at your abdomen in a mirror or by looking down. Do you see a ridge (coning) or a localized bulge (doming) when you lift your head and shoulders? The presence of visible coning or doming suggests that intra-abdominal pressure is pushing out through the weakened midline, which means your core is not able to contain pressure effectively.

Pain or discomfort. Note any pain, discomfort, or pulling sensation during the self-check. Some pulling sensation in the rectus muscles is normal — that is just your muscles contracting. But sharp pain or localized pain at the midline could indicate a hernia or other issue, and you should see a doctor.

How to Know If It Is Improving

Checking yourself once is a snapshot. Checking yourself regularly — every four weeks — gives you a trend line. That trend line tells you whether your core work is working.

When you see improvement, it will usually show up in this order:

1. **Tissue quality improves first.** The tissue at the midline feels firmer and more resilient, even if the width has not changed much yet.
2. **Depth decreases.** The gap becomes shallower. You cannot press as far before hitting resistance.
3. **Coning/doming reduces.** Your abdomen looks flatter when you engage your core. The ridge or bulge becomes less pronounced.
4. **Width narrows.** The gap between the rectus muscles becomes smaller. Your finger-width measurement goes down.
5. **Symptoms improve.** Back pain, incontinence, and other symptoms start to improve. This often happens gradually and may lag behind the physical measurements.

Tracking instructions. Create a simple tracking log. Every four weeks, record the date, your width measurements at all three points, your assessment of tissue quality (firm, moderate, thin), whether you noticed coning/doming, and any symptoms you are experiencing. Over time, this log will show you what is working and what is not.

If you see no improvement after eight weeks of consistent core work, or if your measurements are getting worse, that is a signal that something needs to change. You may need a different exercise approach, better breathing mechanics, or professional guidance.

Things That Can Affect Your Self-Check

Your self-check results can vary slightly from day to day depending on several factors:

Time of day. Your abdominal wall is tighter in the morning and tends to relax throughout the day. Try to do your self-check at the same time of day for consistent comparisons.

Digestion and bloating. A full stomach or intestinal bloating can push the abdominal wall outward and make the gap seem larger. Check first thing in the morning before eating for the most consistent results.

Hydration. Fascia and connective tissue behave differently depending on hydration levels. Good hydration makes tissues more pliable and easier to assess.

Hormonal cycle. Women who are menstruating or breastfeeding may notice changes in tissue quality and gap width during their cycle or in relation to breastfeeding patterns. Hormones affect connective tissue laxity. If you notice your gap seems wider during certain parts of your cycle, that is normal.

Recent activity. If you have just done a core workout or heavy lifting, your tissues may be temporarily inflamed and the gap may feel different. Do your self-check on a rest day or before your workout, not after.

When to See a Pelvic Floor Physical Therapist

The self-check is a useful tool, but it has limits. You should see a pelvic floor physical therapist if any of the following apply:

- **Persistent gap after six months postpartum.** If you have been doing consistent core work for six months and your gap has not narrowed, a PT can assess whether you are targeting the right muscles and whether there are other factors at play.
- **Symptoms of pelvic organ prolapse.** A feeling of heaviness or pressure in the pelvis, a bulge in the vagina, or a feeling that something is falling out. This requires professional assessment.
- **Urinary incontinence.** Any leaking of urine when you cough, sneeze, laugh, jump, or run. (Some leaking during pregnancy is common and usually resolves afterward. Persistent postpartum incontinence is not normal and is treatable.)
- **Fecal incontinence or urgency.** Difficulty controlling gas or bowel movements. This is a pelvic floor issue and needs professional evaluation.

- **Lower back pain that does not go away.** While back pain is common postpartum, persistent back pain that is not improving with rest and gentle movement could be related to diastasis recti and pelvic floor dysfunction.
- **Pain with sexual activity.** Pain during or after intercourse, especially deep pain, can indicate pelvic floor dysfunction.
- **Difficulty feeling your pelvic floor muscles.** If you cannot feel yourself doing a Kegel, or if you cannot relax your pelvic floor after contracting, you are not getting the full benefit of your core work.
- **You are unsure of your self-check.** If you cannot tell whether you are feeling the gap correctly, or if your measurements seem inconsistent, a professional can show you exactly what to feel for.

A pelvic floor physical therapist will do a more thorough assessment than you can do at home. They will check the width and depth of the gap, the quality of the connective tissue, your pelvic floor function (both strength and relaxation), your breathing mechanics, and your movement patterns. They will also rule out hernias, which can feel similar to diastasis recti but require different treatment.

If you cannot access a pelvic floor PT (they can be hard to find, especially outside major cities), look for a physical therapist who has training in women's health or pelvic health. Many now offer telehealth appointments.

The Gap Does Not Define You

It is easy to get discouraged by a wide gap measurement. Do not let a number define your progress or your worth. Diastasis recti is a structural condition, not a reflection of your fitness, your effort, or your worth as a woman.

The self-check is a tool for information and tracking, not for judgment. Use it to learn, to guide your training, and to celebrate progress. If your gap is narrowing, even by half a finger-width, that is real progress. If your tissue quality is improving, that is progress too. Healing happens in small increments over time.

Toolkit Summary

- **The self-check is done lying on your back, knees bent, head and shoulders lifted** — use two or three fingers to probe the gap at the belly button, two inches above, and two inches below.
- **Measure width (finger-widths), depth (how deep the gap is), and tissue quality (firm vs. squishy)** — these three metrics give you a complete picture of your diastasis.
- **Track your measurements every four weeks** — same position, same technique, same time of day — to see your trend line. Tissue quality and depth improve before width narrows.

- **See a pelvic floor PT for persistent gap after 6 months, prolapse symptoms, incontinence, back pain, pain with sex, or if you are unsure of your self-check** — professional assessment is the gold standard.
- **Use the self-check as a learning tool, not a judgment** — healing happens in small increments, and any improvement matters.

ch04

IF YOU ARE USED TO FITNESS GOALS THAT DELIVER VISIBLE RESULTS IN A MATTER OF WEEKS — STRENGTH GAINS, WEIGHT LOSS, MUSCLE DEFINITION — HEALING DIASTASIS RECTI WILL TEST YOUR PATIENCE. THE TIMELINE FOR CONNECTIVE TISSUE HEALING IS MEASURED IN MONTHS AND YEARS, NOT WEEKS. UNDERSTANDING THIS TIMELINE IS ESSENTIAL NOT ONLY FOR SETTING REALISTIC EXPECTATIONS BUT ALSO FOR KEEPING YOURSELF MOTIVATED WHEN PROGRESS FEELS SLOW.

This chapter covers the three stages of connective tissue healing, what is realistic at each stage, and the factors that speed or slow your progress.

The Three Stages of Connective Tissue Healing

The body heals connective tissue — including the linea alba — through three overlapping stages. These stages are not sharply separated; one flows into the next. But each stage has different implications for what you should and should not be doing.

Stage 1: Acute inflammation (first 6 weeks).

Immediately after the stretching or injury occurs — whether that is childbirth, abdominal surgery, or an episode of heavy strain — the body initiates the inflammatory

response. Blood vessels dilate to bring immune cells to the area. Inflammatory chemicals clean up damaged tissue and prepare the area for repair.

During this phase, the tissue is at its most fragile. The collagen fibers that remain are stretched and weakened. The new fibers that will eventually replace them have not yet been laid down. Any load that pushes the tissue beyond its current capacity — including the intra-abdominal pressure from a crunch, a sit-up, a heavy lift, or even a hard cough — can worsen the injury.

For postpartum women, the first six weeks are when the uterus contracts back down, hormones begin to rebalance, and the body does its most dramatic healing. The linea alba may naturally narrow during this period as the mechanical forces of pregnancy are removed. But the tissue itself is still in the inflammatory phase and should be treated gently.

What to do in the first 6 weeks: - Focus on deep, ribcage-dominant breathing. This helps reestablish the neuromuscular connection with the deep core without creating midline pressure. - Practice gentle posture awareness — standing and sitting with your ribs stacked over your pelvis. - Avoid any exercise that creates visible coning or doming. If you see that ridge, you are putting too much pressure on the midline. - Rest. Your body is doing significant healing work and needs energy for recovery. - If you had a C-section, follow your surgeon's guidelines for activity restrictions.

Stage 2: Proliferation (6 weeks to 6 months).

In the proliferation phase, the body lays down new collagen fibers to fill the gap. These new fibers are disorganized — the body is essentially laying down a patch, not restoring the original structure. The tissue gains some strength during this phase, but it is not yet strong enough to handle heavy loads.

This is the phase where you can start doing gentle core work with good form. The mechanical stimulus from proper breathing and gentle TVA engagement guides the new collagen fibers to align in a functional direction. Without this stimulus, the fibers will still form, but they will be randomly oriented and less able to transmit force.

The proliferation phase is the most important period for conservative treatment of diastasis recti. Most women who heal their diastasis with exercise do the majority of their healing during this phase.

What to do in weeks 6 to 24: - Start the core progression: breathing exercises, TVA engagement in supine, heel slides, dead bugs, side-lying work, glute bridges. - Practice posture regularly throughout the day. Stand, sit, and move with your ribs over your pelvis. - Avoid crunches, sit-ups, planks, and any exercise that causes coning or doming. - Be patient. The gap may not change much in the first few weeks of exercise. Tissue quality often improves before width changes.

Stage 3: Remodeling (6 months to 2 years).

The remodeling phase is the longest and least visible. During this phase, the body gradually replaces the disorganized collagen fibers laid down during proliferation

with organized collagen fibers that are aligned with the direction of normal force. This is when the tissue truly regains its tensile strength.

Think of the proliferation phase as building a scab, and the remodeling phase as replacing the scab with normal skin. The scab does the job — it covers the wound — but it is not as strong or functional as the original tissue. It takes a long time for the body to fully replace the scab with strong, organized tissue.

During remodeling, you can gradually return to more demanding activities, including some of the exercises you may have been avoiding. But even during this phase, you should be cautious about exercises that create high intra-abdominal pressure — heavy lifting, high-intensity core work, and extreme ranges of motion in spinal flexion.

Full remodeling of the linea alba can take up to two years. Your connective tissue will continue to strengthen during this entire period, even after the gap has closed enough that you no longer think about it.

What Is Realistic at Each Stage

Knowing the healing stages helps you set realistic expectations.

At 6 weeks postpartum. If you had a vaginal birth, the gap may have narrowed naturally. Studies show that a significant number of women see some natural closure in the first eight weeks. For many, the gap at this point is still present but may be smaller than it was immediately after

birth. If you had a C-section, the incision itself creates additional healing requirements, and you should not be doing active core work until cleared by your surgeon.

At 3 months. If you have been doing consistent deep core work (breathing, TVA engagement, the exercises described here), you may start to see subtle changes. The tissue at the midline may feel firmer. The gap may have narrowed slightly. You may notice that coning or doming is less pronounced. But at three months, you are still in the early stages of healing. Do not expect dramatic results.

At 6 months. This is where many women see the most noticeable improvement. After three to six months of consistent work, the gap has often narrowed by one to three finger-widths. Tissue quality has improved. Coning may be significantly reduced. If you have stuck with the program, this is when you start to feel like your core is working again.

At 12 months. Many women have achieved significant gap reduction or complete closure by twelve months. But "complete closure" — meaning the rectus muscles sit close together with no palpable gap — is not the only marker of success. Some women will always have a small gap (less than two finger-widths) but have no symptoms. They can lift, run, and exercise without pain or dysfunction. That is a successful outcome.

At 18 to 24 months. If your gap was wide (three or more finger-widths) or if you have had multiple pregnancies, complete closure may take up to two years. The connective tissue continues to strengthen and remodel during this entire period. Progress may be very slow in the second year, but it is still happening at the tissue level.

Complete closure is not guaranteed — and that is okay. Some women will always have a small gap. If that gap is not causing symptoms and your core functions well, you do not need to obsess over closing it completely. The goal is a functional, pain-free core, not a specific measurement.

Factors That Speed Healing

If you want to maximize your healing progress, these factors matter:

Consistent core work. The single most important factor is doing the right exercises consistently. Five to ten minutes per day of focused deep core work, plus three to four longer sessions per week, produces measurable results over months. Sporadic effort does not.

Good nutrition. Connective tissue is made of collagen, which is made of protein. You need adequate dietary protein to supply the amino acids for collagen synthesis. You also need vitamin C, which is a cofactor in collagen production. Good sources of protein include meat, fish, eggs, dairy, legumes, and soy. Vitamin C is abundant in citrus fruits, bell peppers, berries, and leafy greens.

Some women find collagen peptide supplements helpful for connective tissue healing. The evidence is mixed, but collagen is a safe supplement and many women report faster healing with it. Talk to your doctor or a nutrition professional about whether supplementation is right for you.

Avoiding pressure on the midline. Every day, you make dozens of micro-decisions about how you move. Do you use your abs to sit up from lying down, or do you roll to your side and use your arm? Do you carry your baby on one hip or centered on your chest? Do you sit with a rounded back or with your ribs stacked over your pelvis? Each of these choices either puts pressure on the midline or spares it. Over weeks and months, these choices add up.

Good posture. Posture is not just about appearing tall and confident. Posture is the position of your skeleton, and your skeleton determines the resting length of your muscles and connective tissue. When you slouch — ribs collapsed, pelvis tucked, head forward — you hold your rectus muscles in a shortened position in front and a lengthened position in back. This creates uneven tension on the linea alba. Good posture — ribs stacked over pelvis, neutral spine — creates even tension that supports midline healing.

Pelvic floor health. The pelvic floor is the bottom of the core canister. If your pelvic floor is weak or too tight, it cannot support the healing process. The deep core system works as a unit — the diaphragm, the transverse abdominis, the multifidus, and the pelvic floor must coordinate to manage intra-abdominal pressure. If any part of this system is dysfunctional, the midline will take longer to heal. A later section covers the pelvic floor connection in depth.

Sleep and stress management. Connective tissue healing happens during rest, not during exercise. Your body repairs itself when you sleep and when you are in a relaxed state. Chronic stress keeps cortisol elevated, which can

impair collagen synthesis and slow healing. Prioritizing sleep and finding ways to manage stress are not luxuries — they are part of the treatment plan.

Factors That Slow Healing

Just as certain factors speed healing, others slow it or actively reverse progress.

Returning to crunches, sit-ups, and planks. This is the most common reason diastasis recti does not heal. Traditional ab exercises create high intra-abdominal pressure that pushes the midline apart. If you are doing crunches or planks, you are working against your own healing. This cannot be emphasized enough. Avoid these exercises until your gap is closed and your core is functioning well.

Heavy lifting too soon. Heavy lifting without proper core engagement — whether that is weightlifting at the gym or lifting a heavy toddler, car seat, or grocery bag — creates the same problem. The intra-abdominal pressure pushes against the midline and widens the gap. The solution is not to avoid lifting (life requires lifting) but to learn to brace your core correctly before each lift. A later section covers this in detail.

Poor breathing mechanics. Belly breathing — expanding the abdomen outward on the inhale — sends pressure directly into the midline. If you breathe this way during exercise or throughout the day, you are putting constant pressure on the connective tissue you are trying to heal. Retraining your breath is one of the most impactful things you can do for your diastasis.

Diastasis left untreated through subsequent pregnancies. If you do not heal your diastasis before getting pregnant again, the second pregnancy will stretch the already-weakened midline further. The separation will be wider, the tissue will be thinner, and healing will take longer. This is one reason why many women notice symptomatic diastasis for the first time after their second or third baby.

Poor nutrition. A diet low in protein and vitamin C means the body lacks the raw materials to build new collagen. If you are not eating enough to support tissue repair, your healing will be slower.

Chronic constipation and straining. Constipation creates repeated intra-abdominal pressure as you strain to pass stool. This pressure pushes against the midline, the same way crunches and heavy lifting do. Managing constipation through diet, hydration, and — if needed — professional guidance is an important part of diastasis treatment.

Smoking and excessive alcohol. Smoking impairs blood flow and oxygen delivery to tissues, which slows healing. Alcohol interferes with collagen synthesis and nutrient absorption. Neither is compatible with optimal connective tissue repair.

Measuring Progress Beyond the Gap

The width of your gap is not the only measure of success. In fact, it may not even be the most important one. Consider tracking these indicators as well:

- **Do you have less back pain?** A healed core reduces strain on the lumbar spine.
- **Can you engage your core without coning or doming?** This is a functional milestone.
- **Do you feel stronger in daily activities?** Lifting, carrying, and moving should feel more controlled.
- **Is your pelvic floor function improving?** Less leaking, better control, more awareness.
- **Do you have more energy at the end of the day?** A functioning core is metabolically efficient. A dysfunctional core wastes energy on compensations.

These markers tell you that your core is healing, even if the gap measurement changes slowly.

Toolkit Summary

- **Connective tissue healing has three phases:** acute inflammation (first 6 weeks — tissue is fragile, be gentle), proliferation (6 weeks to 6 months — new collagen forms, respond to gentle loading), and remodeling (6 months to 2 years — collagen reorganizes, tissue gains strength).

- **Most women see significant gap reduction in 3-6 months of consistent work** — complete closure may take 12-24 months, and some women will always have a small gap without symptoms.
- **Factors that speed healing:** consistent core work, good nutrition (protein and vitamin C), avoiding midline pressure, good posture, pelvic floor health, and adequate sleep.
- **Factors that slow healing:** crunches/sit-ups/planks, heavy lifting too soon, poor breathing mechanics, untreated diastasis through subsequent pregnancies, poor nutrition, chronic constipation, smoking, and excessive alcohol.
- **The goal is a functional, pain-free core, not a specific gap measurement** — track multiple markers of progress, not just the gap width. Talk to your PT about realistic expectations for your specific case.

ch05

BEFORE YOU DO A SINGLE CRUNCH-FREE CORE EXERCISE, YOU NEED TO FIX YOUR FOUNDATIONS. BREATHING, POSTURE, AND INTRA-ABDOMINAL PRESSURE MANAGEMENT ARE NOT OPTIONAL EXTRAS IN DIASTASIS RECTI TREATMENT. THEY ARE THE TREATMENT. THE EXERCISES FOR YOUR CORE WORKOUT WILL NOT WORK IF YOU ARE BREATHING INTO YOUR BELLY, SLOUCHING THROUGH YOUR DAY, AND LETTING YOUR INTRA-ABDOMINAL PRESSURE PUSH THROUGH YOUR MIDLINE.

This chapter covers the three foundational skills that everything else builds on.

The Breath Foundation

Breathing is the most fundamental movement pattern your body performs. You do it about twenty thousand times per day. And every single breath either supports your core healing or works against it.

Ribcage breathing versus belly breathing.

Most people breathe into their belly. When they inhale, they push their abdomen outward. When they exhale, they let it fall back. This is not wrong for everyone — diaphragmatic breathing into the belly is a common relaxation technique, and it is normal during sleep.

But for someone with diastasis recti, belly breathing is problematic. When you push your belly outward on the inhale, you are creating intra-abdominal pressure that pushes forward against the midline. Over twenty thousand breaths per day, that pressure adds up. It keeps the linea alba in a stretched state and prevents the rectus muscles from coming back together.

Ribcage breathing — expanding the ribcage laterally (to the sides) and posteriorly (into the back) — avoids this pressure. Instead of pushing the belly forward, you direct the breath into the lower ribcage, which expands like an accordion. The abdominal contents move downward and outward, not forward. The midline is not pushed from behind.

For diastasis recti, ribcage breathing is the foundation exercise.

How to breathe into your ribcage and back.

Find a comfortable seated or supine position. Place one hand on your lower ribcage (the sides of your ribs, just above your waist) and one hand on your belly.

Inhale through your nose. Direct the breath into your ribcage — feel your lower ribs expand sideways and backward into your hands. Your belly should remain relatively still. It is not completely flat (your abdominal contents have to go somewhere), but the expansion should be in the ribcage, not the belly.

Exhale through your mouth or nose — whichever feels more natural. Let your ribcage close like an accordion. Feel your ribs come back together.

This may feel unnatural at first. Most people have been belly breathing for their entire lives, and retraining requires conscious practice. Start with one minute, three times per day. Gradually increase as it becomes more natural.

Exhale engagement.

The exhale is where the real core work happens. When you exhale fully, your diaphragm rises, your transverse abdominis (the deep corset muscle) engages automatically, and your intra-abdominal pressure decreases. This is the moment of maximal midline safety.

To add core engagement to your exhale: - Exhale fully, emptying your lungs completely. - At the end of the exhale, gently draw your navel toward your spine. This is a small, subtle movement — you are not sucking in your stomach or holding your breath. - Hold that engagement for a second or two, then inhale and release.

This "exhale engagement" is the most basic core exercise for diastasis recti. It is safe to do in any position — lying down, sitting, standing — and you can do it throughout the day. Be careful not to hold your breath while engaged. The pattern is: inhale (ribcage expands), exhale (ribs close, navel draws toward spine), pause (one to two seconds), inhale and release.

The 360-degree breath.

The 360-degree breath takes ribcage breathing a step further: you expand your breath into the front, sides, and back of your torso equally.

Imagine your torso as a cylinder. When you inhale, the cylinder expands in all directions — not just forward into your belly. The front expands into your ribcage. The sides expand into your lower ribs. The back expands into your lower ribcage and the muscles on either side of your spine.

The 360-degree breath is the gold standard of breathing for diastasis recti. It distributes intra-abdominal pressure evenly around the core canister instead of concentrating it on the midline. It also activates the posterior core muscles — the multifidus and the deep spinal stabilizers — which are often neglected in traditional core training.

To practice the 360-degree breath: - Lie on your back with your knees bent. Place one hand on your lower ribcage and one hand on your lower back. - Inhale. Feel your ribcage expand sideways into your front hand and your lower back expand into your back hand. - Exhale. Feel both hands return to their starting position. - Once you can feel expansion in front and back, practice it in seated and standing positions.

Posture

If breathing is the first foundation, posture is the second. Posture is not about standing up straight to look good. Posture is the position of your skeleton, and your skeleton determines the resting length and tension of your muscles and connective tissue.

Why slouching widens the gap.

When you slouch — rounded shoulders, collapsed ribcage, tucked pelvis, head forward — you change the geometry of your core. The rectus abdominis is held in a shortened, slack position. The linea alba loses tension. The transverse abdominis cannot engage properly because the ribcage is collapsed onto the pelvis.

Slouching also affects breathing. A collapsed ribcage cannot expand fully during inhalation. Without full ribcage expansion, you default to belly breathing — which, as discussed above, pushes pressure into the midline.

Posture correction is not about holding a rigid "military" position. It is about finding the position where your skeleton is aligned so your muscles can work at their optimal length.

Standing posture for diastasis.

The ideal standing posture for diastasis recti is simple: ribs stacked over pelvis, neutral spine.

Find your alignment: 1. Stand with your feet hip-width apart, weight evenly distributed. 2. Let your arms hang naturally at your sides. 3. Gently lift the crown of your head toward the ceiling — not pulling your chin up, just lengthening your neck. 4. Let your shoulder blades relax down your back — not pinched together, not pulled forward. 5. Think of your ribcage as a bucket of water. If you tip the bucket forward (ribs flaring up), the water spills out the front. If you tip it backward (ribs tucked down), the water spills out the back. Your ribs should be level — the water stays in the bucket. 6. Your pelvis is in neutral — not tucked

under (posterior tilt) and not arched forward (anterior tilt). Neutral pelvis feels like your hip bones and pubic bone are roughly in the same plane.

In this position, your rectus abdominis is at its optimal resting length. The linea alba has even tension from top to bottom. Your transverse abdominis can engage. Your breathing can expand into your ribcage and back.

Sitting posture for feeding and carrying.

Posture does not stop when you sit down. If you are a new parent, you spend a significant portion of your day sitting — feeding your baby, holding them, watching them play on the floor.

Common sitting mistakes: - Rounding your lower back and letting your belly collapse forward. - Leaning to one side to support the baby on your arm. - Sitting on a soft surface that lets your pelvis sink into an unstable position.

Better sitting posture: - Sit on a firm, flat surface when possible. If you are nursing, a supportive chair with good back support is ideal. - Your hips should be slightly higher than your knees. If you are sitting on a couch, use a cushion or pillow to elevate your hips. - Your ribs should be stacked over your pelvis — same alignment as standing. - Your baby should be brought to your breast or bottle, not the other way around. Hunching over your baby to feed them rounds your back and compresses your core.

If you are bottle feeding or holding your baby while sitting, the same principles apply: keep your ribs stacked, bring the baby to you rather than leaning toward them, and support your arms with pillows so your shoulders do not round forward.

Intra-Abdominal Pressure Management

Intra-abdominal pressure (IAP) is the pressure inside your abdominal cavity. It is created by your diaphragm pushing down when you inhale and by your abdominal muscles and pelvic floor engaging to contain that pressure.

IAP is not inherently bad. It is a necessary part of core function — it stabilizes your spine when you lift something, it helps you cough and sneeze, and it plays a role in everything from childbirth to digestion. The problem is not IAP itself. The problem is IAP that is not properly managed — pressure that is directed forward into the weakened midline instead of contained evenly around the core canister.

The core canister concept.

Think of your core as a canister: the diaphragm is the lid on top, the pelvic floor is the bottom, the transverse abdominis wraps around the sides, and the multifidus stabilizes the back. This is the 360-degree core system.

When this canister works correctly, the diaphragm and pelvic floor move together. On the inhale, the diaphragm descends, the pelvic floor descends slightly, and the

transverse abdominis stretches to accommodate the increase in volume. On the exhale, the diaphragm rises, the pelvic floor lifts, and the transverse abdominis contracts.

This coordinated movement distributes intra-abdominal pressure evenly around the canister. No single wall takes all the pressure. The midline — the linea alba — is protected because the pressure is balanced.

When the canister is disrupted — because of poor breathing, weak pelvic floor, or poor posture — pressure concentrates on the weakest wall. For someone with diastasis recti, that weakest wall is the midline. The pressure pushes into the gap and widens it.

How to manage pressure during daily activities.

Managing intra-abdominal pressure is not something you do once and forget. It is a skill you practice throughout your day, with every movement.

The basic formula for pressure management: 1. **Breathe.** Before you lift, strain, or exert, take a breath into your ribcage. 2. **Brace.** On the exhale, gently engage your deep core — imagine zipping up the front of your abdomen from the pubic bone to the ribcage. This is not a sucking-in motion. It is a gentle, 360-degree tightening of the core canister. 3. **Move.** Exert on the exhale. Lift, carry, or move while maintaining that gentle engagement. 4. **Release.** Inhale and release the engagement. Do not hold your core tight continuously — that would fatigue the muscles and interfere with breathing.

This breath-brace-move-release pattern is the foundation of safe movement for diastasis recti.

Bracing versus bearing down.

There is a critical difference between bracing your core (good) and bearing down (bad).

Bracing is the controlled engagement of the deep core system. It involves the transverse abdominis, the pelvic floor, and the diaphragm working together to create even pressure around the canister. Bracing feels like preparing to receive a punch in the stomach — a gentle, 360-degree tightening. It does not involve holding your breath.

Bearing down is holding your breath and pushing your breath against a closed airway — like you are bearing down to have a bowel movement or to lift something very heavy. Bearing down creates high, uncontrolled pressure that concentrates on the midline. This is the Valsalva maneuver, and it is the enemy of diastasis healing.

If you find yourself holding your breath and pushing during exercise or daily life, stop. Return to ribcage breathing. Reset your core engagement. Then try again with a controlled exhale.

Your breath and pressure check-in.

Throughout the day, do a quick breath and pressure check:

- Are you breathing into your belly or your ribcage?
- Is your posture stacked or slumped?
- Are you bracing or bearing down during exertion?
- Do you feel any coning or doming when you move?

These micro-checks take five seconds and over time become automatic. They are the foundation of every safe movement you will make from now on.

Toolkit Summary

- **Ribcage breathing is the foundation** — expand your lower ribs sideways and backward on the inhale, not your belly forward. Practice the 360-degree breath in supine, seated, and standing positions.
- **Exhale engagement activates the deep core** — at the end of a full exhale, gently draw the navel toward the spine without holding your breath. Inhale and release.
- **Good posture is ribs stacked over pelvis** — in standing and sitting. Slouching shortens the rectus muscles, reduces TVA engagement, and widens the gap.
- **Intra-abdominal pressure is not bad** — uncontrolled pressure that pushes into the midline is. Use the breath-brace-move-release pattern for all daily activities.
- **Brace (360-degree controlled engagement), never bear down (holding your breath and pushing)** — bearing down widens the gap. Practice pressure management throughout the day, not just during exercise.

ch06

THE EXERCISES IN THIS CHAPTER ARE THE HEART OF THE PROGRAM. THEY ARE DESIGNED TO REBUILD YOUR DEEP CORE SYSTEM FROM THE INSIDE OUT — STARTING WITH THE BREATH AND TRANSVERSE ABDOMINIS (TVA) ACTIVATION, PROGRESSING THROUGH GENTLE MOVEMENT PATTERNS, AND EVENTUALLY BUILDING TO FUNCTIONAL STANDING EXERCISES.

These exercises are safe for women with diastasis recti because they avoid putting pressure on the midline. They work the deep core system instead of the superficial rectus abdominis. They emphasize control over range of motion. And they can be modified if you feel coning or doming.

Go back and read the foundations section first. The breathing and posture foundations are prerequisites for this workout. You cannot do these exercises correctly without proper ribcage breathing and intra-abdominal pressure management.

The Safe Core Progression

The exercises are organized as a progression. Start at the beginning and master each level before moving on. Do not skip ahead — the early exercises are not "too easy." They are the foundation that makes the harder exercises safe.

Level 1: Breathing and TVA engagement in supine.

The first level is purely about neuromuscular reeducation — teaching your brain to find your TVA again.

Position: Lie on your back, knees bent, feet flat, arms at your sides.

Exercise 1: Ribcage breath (1 minute). - Inhale into your lower ribcage. Feel your ribs expand sideways and backward. - Exhale fully. Feel your ribs close and your navel draw slightly toward your spine. - Do not force the navel movement. It should happen naturally as a result of full exhalation.

Exercise 2: Exhale engagement (1-2 minutes). - Same position. Inhale into your ribcage. - Exhale fully. At the end of the exhale, gently draw your navel toward your spine. This is a subtle movement — imagine zipping up the front of your abdomen from your pubic bone to your ribcage. - Hold the engagement for 2 seconds. Do not hold your breath. - Inhale and release. Repeat.

Exercise 3: TVA holds (2-3 minutes). - Inhale into your ribcage. - Exhale and engage your TVA — draw the navel toward the spine. - Maintain the engagement through a full inhale-exhale cycle. Breathe into your ribcage while keeping the lower abdominals lightly engaged. - This is harder than it sounds. Most people lose the engagement as soon as they inhale. Practice maintaining the connection through a few breath cycles.

Focus on quality over quantity. Ten good breath cycles with TVA engagement are better than fifty rushed ones with poor form.

Level 2: Heel slides.

Position: Lie on your back, knees bent, feet flat, arms at your sides.

- Inhale into your ribcage.
- Exhale, engage your TVA, and slowly slide one heel forward along the floor until your leg is nearly straight.
- Inhale and hold the extended position, maintaining core engagement.
- Exhale and slide the heel back to the starting position.
- Repeat on the other side.
- Goal: 8-12 reps per side, 2-3 sets.

Heel slides challenge your core in a lengthened position. Your TVA must work to stabilize your pelvis and lower back as your leg extends. If you feel your lower back arching or your belly coning, you have slid too far. Shorten the range of motion until you can maintain engagement.

Level 3: Marching.

Position: Lie on your back, knees bent, feet flat, arms at your sides.

- Inhale into your ribcage.
- Exhale, engage your TVA, and lift one foot about an inch off the floor, keeping your knee bent at 90 degrees.
- Inhale and hold the position, maintaining core engagement.
- Exhale and lower your foot back to the floor.
- Repeat with the other foot.

- Goal: 8-12 reps per side, 2-3 sets.

If you feel your lower back arching when you lift your foot, the leg is too heavy for your current core strength. Decrease the height of the lift — just lifting a toe off the floor is enough. Do not let your back arch.

Level 4: Dead bug.

Position: Lie on your back, knees bent, arms extended toward the ceiling. Lift your feet off the floor so your hips and knees are at 90 degrees.

- Inhale into your ribcage.
- Exhale, engage your TVA, and slowly extend one leg toward the floor while simultaneously extending the opposite arm overhead. Keep your foot and hand barely above the floor — do not touch down.
- Inhale and pause at the extended position.
- Exhale and return to the starting position.
- Repeat on the opposite side.
- Goal: 6-10 reps per side, 2-3 sets.

The dead bug is a more advanced core stabilizer because your limbs are moving away from center, creating a longer lever that challenges your core more. If you feel your lower back arching or your belly coning, reduce the range of motion — move your leg and arm less far from center. You can also keep your feet on the floor (heels down) to make it easier.

Level 5: Side-lying work.

Position: Lie on your side with your legs stacked, knees slightly bent, head resting on your extended arm or a pillow.

Exercise 1: Side-lying hip lifts (clamshells). - Keeping your feet together, lift your top knee toward the ceiling. - Do not let your pelvis roll backward. - Lower with control. - Goal: 10-15 reps per side, 2-3 sets.

Exercise 2: Side-lying leg lifts. - Keeping your leg straight, lift your top leg to hip height. - Keep your torso stable — do not lean backward or forward. - Lower with control. - Goal: 8-12 reps per side, 2-3 sets.

Side-lying exercises strengthen the glute medius and the lateral core, which are important for pelvic stability and balanced core function. Many women with diastasis recti have weak lateral core muscles because of the forward-focused demands of pregnancy and carrying.

Level 6: Glute bridge.

Position: Lie on your back, knees bent, feet flat, arms at your sides.

- Inhale into your ribcage.
- Exhale, engage your TVA, and press through your heels to lift your hips toward the ceiling. Squeeze your glutes at the top.
- Do not let your lower back arch. Your body should form a straight line from your shoulders to your knees.
- Inhale and hold at the top, maintaining core engagement.
- Exhale and lower your hips with control.
- Goal: 10-15 reps, 2-3 sets.

The glute bridge is excellent for diastasis recti because it strengthens the posterior chain (glutes, hamstrings, back extensors) and the deep core without putting pressure on the midline. Many women feel their core working more effectively after a glute bridge than after any ab-targeted exercise.

Level 7: Bird dog.

Position: Start on your hands and knees. Your hands should be directly under your shoulders and your knees directly under your hips. Keep your spine in a neutral position — not rounded, not swaybacked.

- Inhale into your ribcage.
- Exhale, engage your TVA, and slowly extend one arm forward and the opposite leg backward. Keep your arm and leg parallel to the floor.
- Your torso should remain completely still. Do not let your hips rotate or your lower back arch.
- Inhale and pause at the extended position.
- Exhale and return to the starting position.
- Repeat on the opposite side.
- Goal: 6-10 reps per side, 2-3 sets.

Bird dog is a fundamental core stabilization exercise because it challenges your ability to keep your torso stable while your limbs move. If you feel your hips rotating, your lower back arching, or your belly coning, start with just an arm extension (no leg) or a leg extension (no arm). Build up to the full bird dog only when you can maintain perfect stability.

Knees-down bird dog (with knees on the floor and hands on a bench or step) is also an option if the full position is too challenging.

Level 8: Standing core and functional movement.

Once you have mastered the supine exercises, you can progress to standing work.

Heel drop: - Stand with your feet hip-width apart, ribs stacked over pelvis. - Inhale into your ribcage. - Exhale, engage your TVA. - Slowly rise onto your toes, then drop your heels abruptly (a small drop, not a jump). - Catch the landing with your core. Your core should brace automatically on impact. - Goal: 10-12 reps, 2-3 sets.

Squat with core engagement: - Stand with feet hip-width apart. Inhale into your ribcage. - Exhale, engage your TVA, and lower into a squat — as if sitting back into a chair. Keep your chest up and your ribs stacked over your pelvis. - Inhale at the bottom of the squat. - Exhale and stand up, maintaining TVA engagement throughout. - Goal: 10-15 reps, 2-3 sets.

Dead bug variations in standing. As you progress, you can do walking lunges, single-leg balance, and other functional movements while maintaining core engagement. The goal is to carry the deep core connection into everything you do.

Proper Form Cues

These cues will help you stay safe throughout the progression:

Draw your ribs toward your pelvis. Throughout every exercise, think about closing the space between your ribcage and your pelvis. This prevents rib flare — which is when the lower ribs push forward and outward — and keeps the TVA in an optimal position.

Zip up your deep core like a zipper. Imagine a zipper running from your pubic bone up to your sternum. On the exhale, zip it up. On the inhale, the zipper stays closed — do not let it gap open. This visual helps you maintain TVA engagement through full breath cycles.

Exhale on effort. The exhale should coincide with the hardest part of the movement — sliding your heel out, lifting your leg, extending your arm and leg. Exhaling releases pressure in the core canister, creating a safer environment for the midline.

Keep coning and doming from happening. This is your number one safety rule. If you see a ridge (coning) or a bulge (doming) at the midline during any exercise, stop. Reduce the range of motion or the load. Coning and doming mean your core cannot contain the intra-abdominal pressure, and the pressure is escaping through the weakened midline. That is the exact opposite of what you want.

If coning or doming persists at the minimal range of motion, drop back to a lower level of the progression and spend more time on breathing and TVA engagement.

Frequency

The right frequency depends on your schedule and your starting point.

Daily TVA engagement practice: 5-10 minutes per day. This is your "breath and engagement" practice — ribcage breathing, exhale engagement, and TVA holds in supine. This practice keeps the neuromuscular connection alive and builds the habit of ribcage breathing throughout your day.

Full core workout: 3-4 times per week. A full workout includes 5-10 minutes of breath work (Level 1), followed by your current level exercises (2-3 sets each of 2-3 exercises), followed by glute bridges and bird dog at the end. The workout should take 20-30 minutes.

Rest days: On rest days, do your 5-10 minute daily practice. Do not push through fatigue. Your body repairs connective tissue during rest, not during exercise. If you feel sore, that is a sign you did enough.

Listen to your body: If you feel any sharp pain, coning, or doming during any exercise, back off. The exercises should feel like gentle effort — a slight burning in the muscles you are targeting — not pain or strain at the midline.

When to Progress

Progress to the next level when you can complete your current exercises with:

- No coning or doming.
- Full TVA engagement maintained throughout the movement.
-

Controlled breathing (not holding your breath). - Good alignment (no rib flare, no arched lower back, no hip rotation).

If you can do all of that, you are ready to try the next level. Start with fewer reps and shorter range of motion, and watch carefully for coning or doming. If it appears, go back to the previous level for another week or two.

Toolkit Summary

- **Start with breathing and TVA engagement in supine** — master Level 1 before moving to heel slides, marching, dead bugs, side-lying work, glute bridges, bird dog, and standing core exercises.
- **Key form cues:** draw ribs toward pelvis, zip up deep core like a zipper, exhale on effort, prevent coning/doming at all times.
- **Do daily TVA engagement practice (5-10 min) and a full core workout (20-30 min, 3-4x per week)** — rest days are as important as workout days.
- **Avoid these contraindicated movements:** crunches, sit-ups, planks, twisting exercises like Russian twists, full pushups, toe touches, burpees, and any exercise that causes coning or doming.
- **Progress slowly and watch for coning/doming** — if you see it, reduce range of motion or drop to a previous level. Talk to your PT about when you are ready for more advanced exercises.

ch07

IF YOU HAVE BEEN TOLD TO "JUST DO MORE CRUNCHES" TO FIX YOUR POOCH, YOU HAVE BEEN GIVEN DANGEROUS ADVICE. THE TRADITIONAL AB EXERCISES THAT DOMINATE FITNESS CULTURE — CRUNCHES, SIT-UPS, PLANKS, LEG RAISES, RUSSIAN TWISTS — ARE NOT JUST INEFFECTIVE FOR DIASTASIS RECTI. THEY ACTIVELY WIDEN THE GAP. THEY STRENGTHEN THE WRONG MUSCLES. AND THEY CREATE INTRA-ABDOMINAL PRESSURE PATTERNS THAT PUSH THE MIDLINE FURTHER APART.

This chapter explains why these exercises are harmful and gives you specific substitutions that work your core safely and effectively.

Why Crunches and Sit-Ups Are Harmful for Diastasis

Crunches and sit-ups are the most common exercises people reach for when they want to "fix" their belly. They are intuitive — you curl your torso forward, you feel your abs working, and you assume that feel means you are making progress. For diastasis recti, that feeling is deceptive.

They create intra-abdominal pressure that pushes the midline apart.

When you do a crunch or sit-up, you create a significant amount of intra-abdominal pressure. Your abdominal muscles contract, your diaphragm pushes down, and the pressure

inside your core increases. In a healthy core, this pressure is contained evenly around the canister by the transverse abdominis, the pelvic floor, and the multifidus.

In a core with diastasis recti, the midline is the weakest point of the canister. The pressure pushes into that weakened area. The result is visible coning or doming — and structural widening of the gap that you cannot see because it is happening at the tissue level.

Every crunch is a small step backward in your healing. If you do hundreds of crunches over weeks and months, the cumulative effect is significant gap widening.

They strengthen the superficial rectus without engaging the deep core.

Crunches primarily work the rectus abdominis — the superficial "six-pack" muscle. They do very little for the transverse abdominis, the deep corset muscle that actually closes the diastasis gap. In fact, because crunches create a strong contraction of the rectus abdominis, they can pull the two sides of the rectus away from each other at the midline — the opposite of what you want.

Think of the rectus abdominis as a front-end muscle that generates forward flexion. The transverse abdominis is a deep stabilizer that wraps around your torso like a corset. Healing diastasis recti requires strengthening the TVA, not the rectus abdominis. Crunches strengthen the rectus while bypassing the TVA entirely.

The "six-pack" versus deep core distinction.

This is the most important conceptual shift in diastasis recti training. A visible six-pack is a sign of a developed rectus abdominis with low body fat. It is not a sign of a healthy core. In fact, many women with visible six-packs have diastasis recti — they trained their superficial abs while ignoring the deep core system.

The deep core — TVA, pelvic floor, diaphragm, multifidus — does not show up in the mirror. You cannot see it, flex it, or pose it for Instagram. But it is the system that stabilizes your spine, manages intra-abdominal pressure, and keeps your internal organs functioning properly. It is the system that closes the diastasis gap.

Healing diastasis recti often means letting go of the six-pack goal and embracing a different kind of strength — strength you feel in your back, your pelvic floor, and your ability to move without pain.

Planks and Pushups

Planks and pushups are more controversial in the diastasis recti world than crunches and sit-ups. Some experts say they are safe in advanced stages with perfect form. Others say they should be avoided entirely.

The problem with planks.

A plank is an isometric exercise that requires you to hold your body in a straight line, supported by your arms and toes. The core works to keep your hips from sagging and

your back from arching. This is a legitimate core exercise — but it creates high intra-abdominal pressure that pushes into the midline.

Even with perfect form, many women with diastasis recti see coning during planks. The midline cannot contain the pressure, and the gap widens. This is especially true for women with wider gaps or thin connective tissue.

If you can hold a plank without any coning or doming, and if your gap is less than two finger-widths, a short-duration plank (10-20 seconds) may be safe. But most women with diastasis recti should avoid planks until their gap is significantly reduced.

The problem with pushups.

Pushups create a similar pressure challenge as planks — plus they add the dynamic movement of lowering and pressing. The combination of high pressure and movement makes pushups particularly risky for diastasis recti.

There is a version of the pushup that can be safe: an incline pushup against a counter or wall, with the core engaged and the ribs stacked. But full pushups on the floor, even with good form, create too much midline pressure for most women with diastasis recti.

The advanced stage exception.

If you have been doing the core progression for six months or more, and if your gap is less than two finger-widths, and if you have no coning or doming during planks or pushups, and if you have clearance from your PT — then you may be able to reintroduce planks and pushups.

The key is reintroduction, not return. Start with a short plank (10 seconds) and check for coning. If none, add 5 seconds per week. Stay at 30 seconds maximum. For pushups, start with a high incline (counter height) and gradually lower the incline over months. Never do floor pushups if you still have any coning.

If coning appears at any point, back off to the previous stage. The midline is telling you it is not ready.

What About Pilates and Yoga?

Pilates and yoga are excellent movement practices that can support core recovery — but they also include exercises that make diastasis recti worse. The key is knowing which poses to embrace and which to avoid.

Poses that are generally safe and beneficial:

- **Cat-cow.** This spinal articulation is excellent for mobilizing the spine and engaging the deep core. Move slowly, breathe into your ribcage, and focus on feeling the movement in your spine, not in your belly.
- **Child's pose.** A gentle resting pose that decompresses the lower back. It does not create midline pressure.
- **Glute bridge (Setu Bandhasana).** this pose strengthens the posterior chain and the deep core.
- **Bird dog.** This is a Pilates/yoga staple and one of the best exercises for diastasis recti when done with proper form — TVA engaged, torso stable, no coning.

- **Clamshell and side-lying leg lifts.** These are safe for the lateral core.
- **Pelvic tilts.** Small, controlled tilts of the pelvis in supine position. Gentle and effective for finding TVA engagement.
- **Diaphragmatic breathing (in supine or seated).** When done with ribcage expansion (not belly expansion), this is the single most important practice.

Poses that make diastasis worse and should be avoided:

- **Roll-up.** This is a full sit-up in Pilates clothing. It requires spinal flexion against gravity, which creates high intra-abdominal pressure directed at the midline. Avoid it.
- **Boat pose (Navasana).** This pose requires sustained high intra-abdominal pressure as you balance on your sit bones. Most women with diastasis recti will coning in this pose.
- **Full wheel (Urdhva Dhanurasana).** This deep backbend stretches the abdominal wall and can aggravate diastasis. Avoid it until your core is well-recovered.
- **Plank (Phalakasana).** As discussed above, planks are risky for diastasis recti.
- **Hundred (Pilates).** This exercise involves holding a curled-up position while pumping the arms. The sustained abdominal contraction and breath-holding pattern create harmful pressure.

- **Criss-cross (Pilates).** This twisting curl-up combines spinal flexion with rotation — two movements that stress the midline.
- **Full sit-ups in sun salutations.** Many yoga classes include sit-ups in their flow. Substitute a heel slide or a gentle curl-up instead.

The general rule: any pose that involves curling your torso forward (flexion), especially with your feet held or anchored, is likely harmful. Any pose that creates a sustained high-pressure hold is also risky. If you are unsure, ask your instructor for a modification, or choose a class that is specifically designed for postnatal or diastasis-safe practice.

The Exercise Substitution Guide

The simplest way to keep your core work safe is to swap out harmful exercises for safe alternatives. Here is your substitution cheat sheet:

Avoid This Exercise	Substitute With
Crunches	Dead bugs — same rectus abdominis activation without the midline pressure
Sit-ups (full range)	Heel slides — challenges core stabilization without spinal flexion
Planks (on hands or forearms)	Bird dog with knees down — works the same stabilizers with less midline pressure
Full pushups (on floor)	

Avoid This Exercise	Substitute With
	Incline pushups against counter or wall — maintains pressing strength without excessive pressure
Leg raises (supine)	Marching (on back, feet on floor) — challenges hip flexor and core without spinal flexion
Double leg lowers	Heel slides or single leg extensions — works the same pattern with less load
Russian twists	Side-lying clamshells — works obliques and lateral core safely
Bicycle crunches	Bird dog with arm/leg extension — works rotational stability without rotation
Burpees	Squat with core engagement + standing heel drops — builds power without midline stress
Toe touches (lying on back, legs vertical)	Dead bugs — same leg-over-head pattern with appropriate core loading
Ab crunch machines	TVA engagement holds — free up your gym time for something that actually works
V-ups / Jackknives	Dead bugs on an unstable surface — similar intensity, safe pattern
Roll-ups (Pilates)	Pelvic tilts + gentle cat-cow — spinal articulation without flexion pressure

How to Handle Group Exercise Classes

If you take a fitness class — spin, barre, boot camp, CrossFit, or general group fitness — you will likely be asked to do exercises from the "avoid" list. Here is how to handle it:

- **Know your substitutions.** Before class, mentally prepare your alternatives. When the instructor calls for crunches, you do dead bugs. When they call for planks, you do bird dog.
- **Do not feel pressured to follow the class exactly.** Your core is healing. Your needs are different from the person next to you. There is no shame in doing a different exercise.
- **Change position.** Some exercises can be adapted by changing from supine (lying on back) to standing or side-lying. For example, leg raises on the floor can become standing marches.
- **Reduce range of motion.** Even within safe exercises, reduce the range of motion if you feel any coning or doming. Half-range dead bugs are better than full-range coning.
- **Talk to your instructor.** Most instructors are happy to offer modifications if they know you have a medical condition. You do not need to share details — just say, "I have a core injury and need to avoid flexion exercises."

If a class requires you to do exercises that you cannot modify (some boot camp formats move too fast for modifications), consider whether the class environment is right for you during this healing phase. There are many ways to get a good workout without putting your midline at risk.

The "Feeling" Trap

One of the hardest things about avoiding traditional ab exercises is that safe exercises do not always feel like they are working. A dead bug does not burn the way a crunch does. TVA engagement does not produce the satisfying pump of an ab machine.

That burn, that pump, is the rectus abdominis working. It is meaningless for diastasis healing. Worse, it is a sign that you are working the wrong system.

The feeling of safe core work is subtle. You might feel a deep, low-level engagement in your lower abdomen. You might feel your pelvic floor responding. You might feel more stability in your lower back. Over time, you will feel your core working during daily activities — lifting your baby, carrying groceries, getting out of a chair — without thinking about it.

That functional feeling is the real sign of progress. Not the burn. Not the pump. The quiet, stable, unremarkable feeling of a core that just works.

Toolkit Summary

- **Crunches and sit-ups are the enemy of diastasis healing** — they create high intra-abdominal pressure that pushes the midline apart, and they strengthen the superficial rectus while bypassing the deep core system.
- **Planks and pushups are risky even with good form** — most women with diastasis should avoid them until the gap is nearly closed (under 2 finger-widths) and they have PT clearance.
- **Pilates and yoga have safe and unsafe poses** — embrace cat-cow, child's pose, glute bridge, bird dog, clamshells; avoid roll-up, boat pose, full wheel, plank, hundred, criss-cross.
- **Use the substitution guide:** crunches → dead bugs, sit-ups → heel slides, planks → bird dog with knees down, full pushups → incline pushups with core engagement, and so on.
- **Do not chase the burn** — a deep, quiet core connection that stabilizes you in daily life is the real goal. Talk to your PT before reintroducing any flexion-based exercises.

ch08

THE PELVIC FLOOR IS THE MOST OVERLOOKED AND MISUNDERSTOOD PART OF THE CORE SYSTEM. MANY WOMEN HAVE HEARD OF KEGELS. ALMOST NO ONE HAS BEEN TOLD THAT THE PELVIC FLOOR NEEDS NOT JUST STRENGTH BUT COORDINATION, ENDURANCE, AND — EQUALLY IMPORTANT — THE ABILITY TO RELAX.

Your pelvic floor and your diastasis recti are connected in ways that most fitness programs do not address. A weak pelvic floor makes your diastasis harder to heal. An over-tight pelvic floor makes it harder, too. And neither problem can be solved in isolation — the deep core works as a system, and you cannot fix one part without the others.

The Core Canister

The concept of the "core canister" is the most useful model for understanding how your core works as a system.

Imagine your torso as a cylinder or canister with four walls:

- **The top (the diaphragm).** This is the primary breathing muscle. When you inhale, the diaphragm descends, pushing down on the abdominal contents. When you exhale, it rises, releasing pressure.
- **The bottom (the pelvic floor).** This is a sling of muscles that spans the bottom of your pelvis, supporting your bladder, uterus, and rectum. The

pelvic floor moves in coordination with the diaphragm — descending on the inhale, lifting on the exhale.

- **The front and sides (the transverse abdominis).**

This deep abdominal muscle wraps around your torso like a corset, compressing the abdominal contents and stabilizing the spine.

- **The back (the multifidus and deep spinal muscles).** These small muscles run along your spine and stabilize the vertebrae during movement.

When these four components work together, they create a stable, pressurized canister that protects your spine, supports your internal organs, and allows you to move, lift, and exercise safely. The pressure inside the canister is evenly distributed across all four walls — no single wall takes the full load.

Diastasis recti is a failure of the front wall of the canister. The linea alba has weakened and the rectus muscles have separated, creating a weak point where pressure leaks out. But here is the critical insight: that front-wall weakness is often connected to dysfunction at the bottom of the canister — the pelvic floor.

How the Pelvic Floor and Diastasis Interact

The pelvic floor and the diastasis gap are connected through the pressure management system. If the pelvic floor is weak, it cannot hold pressure from below. The diaphragm

pushes down, the pelvic floor drops, and the pressure goes — where? Through the weakest wall. If your linea alba is weakened, that is where the pressure escapes.

This is why Kegels alone are not the answer, and why just doing core exercises without addressing the pelvic floor will only get you so far.

A weak pelvic floor makes diastasis harder to heal. When your pelvic floor is weak, it cannot support the pressure your core generates during daily activities — lifting, coughing, sneezing, running. That pressure goes into the midline, keeping the gap open. No amount of TVA engagement will fully close the gap if the bottom of the canister is not holding its part of the pressure.

Diastasis recti can also weaken the pelvic floor. The same connective tissue stretching that affects the linea alba can affect the pelvic floor. The hormones of pregnancy soften the entire pelvic connective tissue system. A difficult vaginal birth can stretch or tear the pelvic floor muscles. A C-section, while it spares the pelvic floor from the direct trauma of birth, does not protect it from nine months of pressure, hormonal softening, and the stress of recovery.

The relationship is bidirectional. Fixing one without the other is like trying to inflate a tire with a hole in the sidewall and a hole in the tread. You plug one hole, but the air still escapes through the other.

One study found that women with diastasis recti were significantly more likely to have pelvic floor dysfunction — including urinary incontinence and pelvic organ prolapse — than women without separation. The conditions are linked, and they respond best to linked treatment.

The Kegel Question

Kegels are the most prescribed exercise for postpartum women. And for good reason — they strengthen the pelvic floor muscles, which can improve incontinence and support the pelvic organs.

But Kegels are not a universal good. They can be done incorrectly, excessively, or at the wrong time. And for some women with diastasis recti, Kegels can make things worse.

When Kegels help:

- When you have a weak pelvic floor that cannot generate enough tension to support your core.
- When you have stress urinary incontinence (leaking with coughing, sneezing, laughing, or exercise).
- When you can feel your pelvic floor muscles contracting and relaxing normally.
- When you are doing them as part of a complete core program that includes diaphragmatic breathing and TVA engagement.

When Kegels make things worse:

- When your pelvic floor is already too tight (hypertonic). Some women, especially those who have had a difficult birth, hold their pelvic floor in a

chronically contracted state. Forcing more contractions on top of this can create pelvic pain, pain with sex, and difficulty fully emptying the bladder or bowel.

- When you are doing Kegels with the wrong technique — holding your breath, bearing down, or engaging your glutes and thighs instead of the pelvic floor.
- When you are doing Kegels in isolation without addressing breathing and TVA coordination. The pelvic floor is part of a system. Strengthening it without integrating it into the larger core canister can create more tension, not better function.

The right technique: the "elevator" cue.

Imagine your pelvic floor is an elevator in a building. The floor of the building is your fully relaxed pelvic floor. The top floor — where you want to go during a Kegel — is a gentle, controlled lift.

1. **Inhale.** Inhale into your ribcage. As you inhale, feel your pelvic floor descend slightly — the elevator goes to the ground floor.
2. **Exhale.** On the exhale, gently lift the pelvic floor upward — the elevator goes up one or two floors. Imagine you are stopping a stream of urine mid-flow, but with much less force than that. The movement is subtle: a gentle squeeze and lift.
3. **Hold.** Hold the lift for 2-3 seconds while continuing to breathe normally. Do not hold your breath.

4. **Fully release.** Exhale completely and let the pelvic floor return to its fully relaxed position. The elevator goes all the way back down to the ground floor. Full release is as important as the contraction.

The "elevator" cue is better than the "stop the flow of urine" cue because many women bear down when they try to stop urine flow. The stop-flow cue can become a bearing-down exercise if done incorrectly.

Key Kegel tips: - Do not hold your breath. If you are holding your breath, you are doing it wrong. - Do not squeeze your glutes, thighs, or lower abdominals. The pelvic floor should work independently. - Quality over quantity. Ten perfect Kegels are better than fifty rushed ones. - Always follow a contraction with a full release. A pelvic floor that cannot relax is as dysfunctional as one that cannot contract.

How many and how often:

Start with 5 Kegels, 3 times per day. Each Kegel: lift and hold for 3 seconds, then fully relax for 5 seconds. As you get stronger, increase the hold time to 5-10 seconds and add "quick flicks" — rapid 1-second lifts.

Never Kegel while you are urinating as a regular practice. The stop-flow test is a one-time diagnostic, not a training method. Doing Kegels while urinating can interfere with the bladder's normal emptying and increase the risk of urinary tract infections.

Pelvic Floor Awareness

Before you can strengthen your pelvic floor, you need to be able to feel it. Many postpartum women have lost the connection to their pelvic floor — they cannot feel a contraction, or they cannot tell if they are doing a Kegel correctly.

Finding your pelvic floor.

Next time you use the toilet, try to stop the stream of urine mid-flow. If you can stop it (or slow it significantly), you have contracted your pelvic floor. This is a diagnostic test only — do not do this repeatedly.

Another way to find your pelvic floor: insert a clean finger into your vagina and gently squeeze as if you are doing a Kegel. You should feel a gentle tightening around your finger. If you feel nothing, or if you feel a pushing sensation, your technique needs work.

Contracting and relaxing intentionally.

The ability to contract your pelvic floor is important. But the ability to relax it is equally important. A pelvic floor that is always tight cannot function properly. It cannot coordinate with the diaphragm. It cannot allow for full, comfortable sexual activity. And it can contribute to pelvic pain and difficulty with bowel movements.

Practice this sequence: 1. Inhale. As you inhale, consciously relax your pelvic floor. Let it descend. Feel the "letting go" sensation. 2. Exhale. Gently lift the pelvic floor. Hold for 2-3 seconds. 3. Inhale again. Fully release. Let the floor drop. 4. Repeat.

The ability to move from contraction to full relaxation is the real skill. Many women can contract but cannot fully release. If that sounds like you, spend extra time on the release phase.

The relationship between pelvic floor tension and breath.

Your pelvic floor moves with your breath. On the inhale, the diaphragm descends and the pelvic floor descends. On the exhale, the diaphragm rises and the pelvic floor lifts. This is the natural, healthy rhythm.

If you have a weak pelvic floor, the descent on the inhale may be too much — the pelvic floor drops too far and does not lift well on the exhale. If you have a tight pelvic floor, the descent on the inhale may be reduced — the pelvic floor is holding too much tension to move freely.

Try this: lie on your back, knees bent, feet flat. Place one hand on your lower ribcage and one hand on your pubic bone. Inhale into your ribcage and feel your pelvic floor descend. Exhale and feel it lift. This is your breath-pelvic floor connection. Practice it until you can feel the movement clearly.

When to Work with a Pelvic Floor PT

Some pelvic floor issues cannot be resolved with self-directed Kegels and breath work. You need a professional pelvic floor physical therapist if you have any of the following:

- **Persistent urinary incontinence.** If you are still leaking urine at 6 months postpartum — even a small amount — this is not normal. A pelvic floor PT can assess your pelvic floor strength, coordination, and endurance, and design a targeted program.
- **Symptoms of pelvic organ prolapse.** A feeling of heaviness or pressure in the vagina, a sensation of something "falling out," or actually seeing or feeling a bulge at the vaginal opening. Prolapse is treatable, especially when caught early.
- **Pain with sexual activity.** Pain during intercourse, especially deep pain, can indicate pelvic floor tension or dysfunction. A PT can help you learn to relax the pelvic floor.
- **Difficulty feeling pelvic floor engagement.** If you have been trying Kegels for weeks and cannot feel any contraction, you may have a neuromuscular coordination issue that a PT can address with biofeedback or manual therapy.
- **Diastasis recti that is not improving.** If you have been doing the exercises in this book for 8-12 weeks with no measurable improvement, a pelvic floor PT can assess whether pelvic floor dysfunction is interfering with your core work.

- **Continence issues with gas or stool.** Inability to control gas or bowel movements is a significant pelvic floor dysfunction that requires professional assessment.

Pelvic floor PTs are specialized physical therapists. They do internal assessments (a gloved, lubricated finger inserted into the vagina or rectum) to evaluate pelvic floor muscle function. This assessment is diagnostic and should not be painful. If it is painful, tell your PT — they can adjust their technique.

Many pelvic floor PTs now offer telehealth appointments, which can be helpful if you live in an area without a specialist. They can teach you the exercises and breathing patterns remotely.

Putting It Together: The Coordinated Core

The goal of all this work — the breathing, the TVA engagement, the Kegels, the core exercises — is a coordinated core system. You want your diaphragm, pelvic floor, and TVA to work together smoothly during everything you do.

Here is a simple exercise to practice coordination: - Lie on your back, knees bent. - Inhale into your ribcage. Feel your pelvic floor descend. - Exhale. Gently lift your pelvic floor. Simultaneously, draw your navel toward your spine (TVA engagement). - Hold for 2-3 seconds. Both the pelvic floor lift and the navel draw should happen together. - Inhale and release both together — pelvic floor descends, navel releases. - Repeat.

Once you can do this in supine, practice it seated, then standing, then during movement — like during a squat or a bird dog. The goal is for the coordinated engagement to become automatic. You should not have to think about it every time you lift your baby or pick up a grocery bag. With enough practice, the pattern becomes instinctive.

Toolkit Summary

- **The core canister has four walls:** diaphragm (top), pelvic floor (bottom), TVA (front and sides), multifidus (back). All four must work together to manage pressure and stabilize the core.
- **A weak pelvic floor makes diastasis harder to heal** and vice versa — they are connected through the pressure management system and must be treated together.
- **Kegels help when the pelvic floor is weak** — but they hurt when the pelvic floor is already too tight. The "elevator" cue (lift and hold, then fully release) is the right technique. Never hold your breath.
- **Pelvic floor awareness requires both contraction and relaxation** — the ability to fully release is as important as the ability to contract. Practice the inhale-relax, exhale-lift pattern.
- **See a pelvic floor PT for:** persistent incontinence, prolapse symptoms, pain with sex, difficulty feeling pelvic floor engagement, or diastasis that is not improving with exercise. These issues respond well to professional treatment.

ch09

YOUR CORE WORK DOES NOT STOP WHEN YOU LEAVE YOUR EXERCISE MAT. IN FACT, MOST OF YOUR HEALING HAPPENS DURING YOUR DAILY ACTIVITIES — THE HUNDREDS OF SMALL MOVEMENTS YOU MAKE THROUGHOUT THE DAY THAT EITHER STRESS THE MIDLINE OR SUPPORT IT.

This chapter covers the everyday movements that matter most for diastasis recti: lifting your baby, carrying them through the day, getting out of bed, handling car seats and groceries, and setting up your environment for core-friendly movement.

If you do your exercises perfectly but move poorly in daily life, you are working against yourself. The goal is to integrate safe core mechanics into everything you do.

How to Lift Your Baby

Lifting your baby is the most repetitive movement you will do as a new parent. You lift them from the crib, from the car seat, from the floor, from the bathtub, from the stroller. You lift them when they are sleeping (carefully, so they do not wake up) and when they are thrashing (less carefully). You lift them dozens of times per day.

Each of these lifts is an opportunity to support your core healing — or to stress your midline.

Squat, don't bend.

When you lift anything — your baby, a car seat, a laundry basket — the correct movement is a squat, not a bend. Bending at the waist with straight legs puts the load on your lower back and creates a long lever arm that pulls on your abdominal wall. Squatting — bending at the knees and hips while keeping your torso upright — keeps your core in a stable, supported position.

To lift your baby from a low surface or the floor: 1. Stand close to your baby. Your feet should be hip-width apart. 2. Squat down by bending your knees and hips. Keep your chest up and your ribs stacked over your pelvis. 3. Reach for your baby. Bring them close to your body before you stand up. 4. Exhale and stand up, keeping your baby centered against your chest.

If your baby is in a crib or a bassinet that requires you to reach over a rail, the same principle applies: lower the side rail if possible, or step as close as you can, squat, and keep your baby close to center as you lift.

Exhale on the lift.

This is the single most important breathing cue for lifting. Exhale as you stand up from the squat. The exhale decreases intra-abdominal pressure and engages your deep core. If you hold your breath, you are creating pressure that pushes against your midline.

The pattern is: inhale before you lift, exhale as you lift, inhale to reset.

Keep baby close to center.

The further a load is from your center of gravity, the harder your core has to work. If you lift your baby with your arms extended, your core must generate more tension to stabilize you. If you lift with your baby pressed against your chest, the load is close to center and your core works more efficiently.

Get your baby as close to your body as possible before you begin the lift. If you are lifting them from a car seat, hold the car seat handle and bring it toward your body before lifting it upward.

Brace your core before lifting.

Before you lift, take a moment to brace your core. This is not a deep breath held — it is a gentle, 360-degree engagement of your deep core, the "zipping up" sensation described in the foundations section. Practice bracing before every lift until it becomes automatic.

If you feel any coning or doming during a lift, you are not bracing correctly. Stand back, reset, and try again with a deeper exhale and a stronger brace. If coning persists, you may be loading too much too soon. Find ways to reduce the load — or ask for help with the heavier lifts.

Carrying Your Baby

Once you have lifted your baby, you carry them throughout the day. Some carrying positions are better for your core than others.

Wear your baby centered when possible.

If you use a baby carrier or wrap, position your baby in the center of your chest, not on your hip. A centered position keeps your weight evenly distributed over your core. There is no asymmetric load pulling on the midline.

Use a structured carrier that provides good head and neck support for your baby and has a wide, supportive waistband. The waistband should sit on your hips, not your waist, and should be snug enough to transfer some of the baby's weight to your pelvis rather than your shoulders and core.

When you wear your baby on your front, be aware of your posture. Many women lean backward to compensate for the weight on their front. This arch in the lower back puts the rectus abdominis in a lengthened, stretched position. Keep your ribs stacked over your pelvis. Engage your glutes slightly to support your pelvis in a neutral position.

Avoid hip carrying that shifts weight off-center.

Carrying your baby on one hip is one of the most common core-unfriendly positions. It shifts your weight to one side, creating an asymmetric load on your core and pulling on the midline. If you carry your baby on your left hip for hours a day (which most right-handed people do), you are creating a chronic lateral pull on the linea alba.

If you must carry your baby on your hip (which is sometimes unavoidable), do it equally on both sides. Switch sides frequently. Keep your baby as close to your body as possible. And be aware that this position is not ideal for your core healing.

When your baby is old enough to sit on your hip independently, they may resist being carried in the center. You can still alternate sides and be mindful of your posture.

The toll of heavy toddlers.

As your baby grows into a toddler, the load increases. A 20-pound toddler creates significantly more asymmetric load than a 7-pound newborn. Your core work will need to keep up with your child's growth.

For heavy lifting, use a structured carrier that supports the weight on your hips and keep your child centered. For short carries, use the squat, exhale, brace approach. Avoid carrying a heavy toddler on one hip for long periods.

As your child becomes more independent, they may want to be carried less — which also gives your core more time to rest and heal.

Getting Out of Bed

Getting out of bed seems trivial. But you do it at least once per day, every day, and most people do it in a way that stresses the midline.

The standard approach: lie on your back, sit up with straight legs, swing your legs over the side. This uses the rectus abdominis in a crunch-like movement, pulling the midline apart.

The better approach: the log roll.

The log roll technique:

1. **Bend your knees.** From lying on your back, bend your knees so your feet are flat on the bed.
2. **Roll to your side.** Keeping your knees together, roll your entire body to one side. Your back should stay straight — think of rolling like a log, not twisting at the waist. Use your arm to help you roll if needed.
3. **Use your arm to push up.** From the side-lying position, place your top arm on the bed in front of you (or on the side of the bed) and push yourself up to a seated position. Your legs will naturally drop off the side of the bed as you push up.
4. **Stand up.** Once seated, stand up using your legs, not your back.

The log roll keeps your spine in alignment and avoids the sit-up motion that stresses the midline. It takes a few extra seconds, but it is worth it.

Don't sit up with straight legs or lead with your head.

Two common habits to break: - Sitting up with straight legs. This increases the lever arm and puts more stress on the rectus abdominis. Always bend your knees before you sit up. - Leading with your head. When you sit up by pulling your head forward and curling your shoulders, you are doing a crunch. Lead with your ribs. Keep your neck long and your chin neutral.

Car Seats, Groceries, and Laundry — The Functional Core Approach

Your core work extends to every carrying and lifting task you do.

Car seats with handles. These are heavy and awkward. The handle lengthens your lever arm. Use two hands whenever possible. Lift from a squat, with the car seat close to your body. If you are lifting a car seat with a baby inside, it is heavy — brace your core thoroughly before lifting.

Better: many car seats now have a travel system that lets you attach the seat to a stroller base. Use this feature when you can. It saves your core from the lift.

Groceries. Grocery bags are asymmetric loads. Two bags, one on each side, is more balanced than one bag. If you only have one bag, carry it close to your body and think about keeping your shoulders level — do not hike one side up to compensate for the weight.

If you are carrying a heavy grocery bag, treat it like any other lift: squat, exhale, brace. Do not twist your torso as you lift.

Laundry. Laundry baskets are heavy, and they are often carried in front of the body, which puts pressure on the midline. Carry the basket centered and close to your body. If the basket is too large, consider dividing the load into smaller loads. Better to do two trips with less weight than one trip with too much.

A laundry basket on the floor is a squat challenge: squat down, bring the basket close, brace, exhale, and stand up. Do not bend at the waist.

The two-hand rule. For any lift, use both hands. Asymmetric loading — one hand, one side — produces a rotational force on the core. Two hands, evenly spaced, keeps the load balanced.

The Nursing Station Setup

If you are nursing or bottle feeding, you spend hours per day in a seated position. Your posture during feeding matters for your core health.

The common mistake.

Most women hunch over their baby to feed them. They lean forward, round their shoulders, and let their ribcage collapse onto their pelvis. This rounded posture keeps the rectus abdominis in a shortened position and reduces TVA engagement throughout the feeding session.

The better setup.

Set up your feeding station so you do not have to compromise your posture: - Sit in a supportive chair with your back supported. A nursing chair or a dining chair with a cushion is better than a soft couch. - Your hips should be slightly higher than your knees. If your chair is too low, add a cushion or a folded towel. - Bring your baby to your breast or bottle — do not lean forward to reach them. Use a nursing pillow or a regular pillow to prop your baby up to the right

height. - Keep your shoulders relaxed. Your ribcage should be stacked over your pelvis, not collapsed forward. - Put your feet flat on the floor. If your feet do not reach, use a footrest.

Setting up for the long haul.

Every time you feed your baby, you have the opportunity to practice good core mechanics. Use the time to practice ribcage breathing — it is a perfect opportunity for daily practice. Place one hand on your lower ribs and one on your belly. Inhale into your ribs. Exhale fully. Repeat.

If you are bottle feeding and holding your baby in your arms, switch arms mid-feed to balance the load on your core. Set up a pillow or support under your arm so you do not have to hold your baby's weight with your arm and shoulder muscles alone.

Adapting as Your Baby Grows

The demands on your core change as your baby grows. A newborn weighs 6-10 pounds and spends most of their time lying still. A toddler weighs 25-35 pounds and is in constant motion.

Newborn stage: Focus on breathing, posture during feeding, and gentle lifting from flat surfaces. Your core is in its most fragile state. Do not push it.

4-6 months: Your baby can hold their head up and is becoming more interactive. Your lifting frequency increases. Focus on consistent squat-and-exhale technique.

6-12 months: Your baby is sitting, scooting, and possibly crawling. They are heavier and more wiggly. Core engagement is more important than ever. Be careful with asymmetrical carrying.

Toddler stage: Your child is heavy and active. Squat-and-lift is your primary mechanism. Use structured carriers for longer carries. Be extra careful with car seats and heavy loads.

At every stage, listen to your body. If you feel coning, doming, or back pain during any daily activity, you are loading your core beyond its current capacity. Reduce the load, change the technique, or ask for help.

Toolkit Summary

- **Lift your baby with a squat, not a bend** — keep your chest up, baby close to center, exhale as you lift, brace your core before every lift.
- **Carry your baby centered on your chest** — avoid one-sided hip carrying that creates asymmetric load. Use a structured carrier with a supportive waistband.
- **Use the log roll to get out of bed** — bend your knees, roll to your side, use your arm to push up. Do not sit up with straight legs or lead with your head.
- **Handle car seats, groceries, and laundry with the functional core approach** — two hands, squat to lift, carry loads close to center, divide heavy loads into smaller ones.

- **Set up a supportive nursing station** — bring baby to you, keep ribs stacked, practice ribcage breathing during feeding sessions. Adapt your core approach as your baby grows.

ch10

MOST OF THIS BOOK IS ABOUT TREATING DIASTASIS RECTI WITHOUT SURGERY. AND FOR MOST WOMEN, CONSERVATIVE TREATMENT — BREATHING, TVA ENGAGEMENT, THE CORE PROGRESSION, AND DAILY MOVEMENT MODIFICATIONS — IS ENOUGH TO CLOSE THE GAP AND RESTORE CORE FUNCTION.

But for some women, conservative treatment is not enough. Their gap is too wide, their connective tissue is too thin, or they have associated problems — like a hernia or significant pelvic organ prolapse — that require surgical repair.

This chapter covers when surgery is appropriate, what the procedure involves, and how to find a good surgeon. It is not a recommendation to pursue surgery. It is information to help you make an informed decision if conservative treatment has not worked for you.

When Surgery Is Appropriate

Surgery for diastasis recti — also called abdominoplasty with diastasis repair, or simply abdominal wall reconstruction — is a major procedure. It is not the easy way out, it is not a shortcut, and it is not cosmetic. It is a structural repair of a damaged abdominal wall.

The gap is wider than 2-3 finger-widths and persists after 12+ months of consistent conservative treatment.

This is the most common indication for surgery. If you have been doing the exercises in this book — consistently, with good form, for at least a year — and your gap is still more than two to three finger-widths wide, surgery may be appropriate.

The key phrase is "consistent conservative treatment." You need to have done the work. If you did the exercises sporadically for a few months and gave up, surgery is not the next step. Give conservative treatment a full, dedicated effort first.

Associated symptoms are present.

Surgery is more strongly indicated if your diastasis is causing significant symptoms:

- **Chronic lower back pain.** If your separated core is contributing to persistent back pain that has not responded to physical therapy.
- **Poor core function.** You cannot lift, carry, or exercise without coning, doming, or discomfort. Your core feels weak and unreliable.
- **Pelvic organ prolapse.** Diastasis recti and prolapse are linked. If you have prolapse and significant diastasis, surgical repair of the abdominal wall may be part of your treatment plan.
- **Umbilical hernia.** A hernia at the belly button is common with diastasis recti, especially if the gap is wide. Hernias are a medical condition that may require surgical repair regardless of the diastasis.

Cosmetic concerns that affect quality of life are also valid. Some women find that the appearance of their belly — the pooch, the bulge, the loose skin — affects their self-image and willingness to exercise or be active. This is a legitimate reason to consider surgery. But surgery should not be pursued for cosmetic reasons alone unless you have fully explored conservative options.

Who is a candidate.

Not everyone with a wide gap is a candidate for surgery. Surgeons typically look for these criteria:

- **Completed childbearing.** Most surgeons recommend delaying diastasis repair until you are done having children. A subsequent pregnancy after surgical repair can stretch the repaired midline and undo the surgery. If you are planning more children, continue conservative management and plan for surgery when your family is complete.
- **Stable weight.** Significant weight fluctuations after surgery can affect the results. Surgeons generally prefer you to be at a stable weight for at least 6-12 months before surgery.
- **Completed physical therapy course.** You should have completed at least 6-12 months of physical therapy before considering surgery. This is not just to try conservative treatment first — it is also to strengthen the muscles that will be repaired. Surgery repairs the connective tissue, but the muscles need to be functioning well before the surgery to get the best results.

- **Good general health.** Surgery requires general anesthesia and has a significant recovery period. You need to be healthy enough to tolerate both.
- **No active infection or untreated medical conditions.** Active infections, uncontrolled diabetes, and other medical conditions need to be addressed before surgery.

What Abdominoplasty Requires

Abdominoplasty with diastasis repair is major surgery. It is not a quick fix.

Recovery timeline: 6-8 weeks (initial recovery) to 6 months (full recovery).

The recovery from diastasis repair surgery is similar to recovery from a C-section, but more involved:

- **First 1-2 weeks.** You will need help with daily activities — including getting out of bed, showering, and caring for your children. You will not be able to lift anything heavier than a few pounds. You will need someone to help you with your baby. Plan for this in advance.
- **Weeks 2-4.** You may be able to do light daily activities, but no lifting, pushing, pulling, or exercise. You will still be restricted in how you move. You will be wearing an abdominal binder or compression garment.

- **Weeks 4-6.** You may be cleared for light walking and very gentle movement. No bending, twisting, or lifting. Most people can return to desk work by this point.
- **Weeks 6-8.** You may be cleared for light exercise — walking, gentle stretching, very basic core work (breathing and gentle TVA engagement only).
- **Months 2-6.** Gradual return to normal activities, including lifting and exercise. You will need to rebuild your core strength slowly, starting from the basics.
- **Full recovery.** It can take 6 months to a year to feel fully recovered from the surgery. The scar will continue to mature. The repaired tissue will continue to strengthen.

Surgery repairs the connective tissue, but muscle function requires training.

This is the most important thing to understand about diastasis surgery. The surgery tightens and sutures the linea alba — the connective tissue — back together. This closes the gap. But the surgery does not retrain your transverse abdominis. It does not teach your pelvic floor to coordinate with your diaphragm. It does not fix your breathing pattern or your posture.

After surgery, you will still need to do the core work from this book. The surgery makes the exercises more effective because the connective tissue is now intact. But if you do not do the exercises, the muscles will remain weak and the core will not function properly.

Many women who have diastasis repair surgery and do not follow up with physical therapy or core work end up with recurrent symptoms — not because the surgery failed, but because they did not retrain their muscles. The surgery is structural. The training is functional. You need both.

Cost and insurance coverage.

Abdominoplasty with diastasis repair can cost between \$8,000 and \$20,000 in the United States, depending on your location, the surgeon, and the complexity of the procedure.

Some insurance plans cover the procedure if it is deemed medically necessary — for example, if you have a hernia that requires repair, or if your diastasis is causing significant symptoms. Other plans consider it cosmetic and do not cover it. Check with your insurance provider before scheduling surgery.

If your insurance does not cover it and you need to pay out of pocket, ask about payment plans. Many surgeons offer financing options.

The Procedure Itself

The surgical repair of diastasis recti is called plication of the linea alba.

What happens during the procedure.

Under general anesthesia, the surgeon makes an incision in the lower abdomen — typically a horizontal incision low on the abdomen, similar to a C-section scar but usually longer and lower. The surgeon separates the skin from the underlying muscle and fascia to access the linea alba.

The surgeon then sutures (plicates) the linea alba down the center, bringing the two rectus muscles back together. The sutures are typically placed in multiple layers to create a strong repair. The number of sutures and the technique vary depending on the surgeon's preference and the width of the gap.

Often combined with hernia repair.

If you have an umbilical hernia (a common companion to diastasis recti), the hernia can be repaired at the same time. The surgeon closes the hernia defect and may use mesh to reinforce the repair.

Sometimes combined with liposuction.

Some surgeons recommend liposuction of the upper abdomen to improve the contour and remove excess fatty tissue. Liposuction is generally considered cosmetic and may not be covered by insurance. Discuss whether this is appropriate for you and what the additional cost would be.

General anesthesia and overnight stay.

The procedure requires general anesthesia. You will likely stay in the hospital overnight for monitoring. Some surgeons perform the procedure as an outpatient procedure (you go home the same day), but an overnight stay is common, especially if hernia repair or liposuction is included.

The scar.

The scar from abdominoplasty is significant. It runs horizontally across your lower abdomen, from hip bone to hip bone. It may be hidden by underwear or swimwear, but it is not invisible. The scar will be red and raised initially and will fade over 12-18 months.

Think carefully about whether you are prepared for a visible surgical scar. Some women find the scar an acceptable trade-off for a healed core. Others prefer to avoid the scar and continue with conservative treatment.

How to Find a Good Surgeon

If you decide to pursue surgery, the choice of surgeon is the most important decision you will make. A skilled surgeon with experience in diastasis recti repair will give you a better result and fewer complications than a less experienced surgeon.

Look for board certification.

Your surgeon should be board-certified by the American Board of Plastic Surgery (if choosing a plastic surgeon) or the American Board of Surgery (if choosing a general surgeon who specializes in abdominal wall reconstruction). Board certification means the surgeon has completed the required training and passed rigorous examinations.

Do not choose a surgeon who is not board-certified. This is non-negotiable.

Look for experience with diastasis recti repair.

Ask potential surgeons: "How many diastasis recti repairs have you performed?" A surgeon who has done this procedure many times will be better prepared for variations in anatomy and unexpected challenges.

Look for a surgeon who specializes in abdominal wall surgery, body contouring after pregnancy, or hernia repair. These specialties overlap significantly with diastasis recti repair.

Ask about their technique for plication.

There are different techniques for plication (suturing the linea alba). Some surgeons use a single-layer closure. Others use multiple layers. Some use permanent sutures. Others use absorbable sutures.

There is no definitive evidence that one technique is superior to all others. But a good surgeon should be able to explain their technique and the reasoning behind it. If a surgeon cannot clearly explain what they will do and why, that is a red flag.

What to ask during a consultation.

- "How many diastasis recti repairs have you performed?"
- "What is your technique for plication?"
- "Do you use permanent or absorbable sutures?"
- "Do you repair hernias at the same time?"
- "What is your complication rate?"
- "What does the recovery timeline look like for your patients?"
- "Do you recommend physical therapy before and after surgery?"
- "Can I see before and after photos of your patients?"
- "What is the total cost, including anesthesia and facility fees?"

Get a second opinion.

This is major surgery. Getting a second opinion from another qualified surgeon is not insulting to the first surgeon — it is responsible medical decision-making. A second opinion may confirm the first surgeon's recommendations, or it may offer a different perspective on whether surgery is necessary and which approach is best.

Timing matters.

Even if you are a good candidate for surgery, timing matters. Do not schedule surgery when you are the sole caregiver for young children. Do not schedule it during a stressful period at work or during a time when you cannot take adequate time off for recovery.

Plan your surgery for a period when you have at least 6-8 weeks of reliable support for childcare and daily tasks. Your recovery should be your only job during the initial weeks.

A Note of Encouragement

If you have read this chapter because you have been struggling with a wide gap that has not responded to treatment, know that you are not alone. Some diastases are too wide or too thin to close with exercise alone. This is not a failure on your part. It is a structural limitation that surgery can address.

Surgery is not giving up. It is making a choice to pursue a different path to the same goal: a functional, pain-free core. Whichever path you take, the work of rebuilding your core from the inside continues.

Toolkit Summary

- **Surgery is appropriate when the gap is wider than 2-3 finger-widths after 12+ months of consistent conservative treatment** — and when associated symptoms like back pain, poor core function, or prolapse are present.
- **Good candidates have completed childbearing, stable weight, completed PT, and good general health** — you must do the core work before and after surgery.
- **Recovery takes 6-8 weeks for initial healing and up to 6 months for full recovery** — surgery repairs the connective tissue, but muscle function requires ongoing training. Do not expect surgery to be a shortcut.
- **The procedure involves plication (suturing) of the linea alba** — often combined with hernia repair and sometimes liposuction. General anesthesia and an overnight hospital stay are typical.
- **Find a board-certified plastic or general surgeon with experience in diastasis recti repair** — ask about their technique, how many repairs they have done, and what recovery looks like. Get a second opinion before deciding. Talk to your doctor and PT about whether surgery is right for you.

ch11

YOU HAVE DONE THE WORK. YOUR GAP HAS NARROWED — PERHAPS FULLY CLOSED. YOUR CORE FEELS STABLE. YOU CAN LIFT, CARRY, EXERCISE, AND MOVE WITHOUT PAIN OR CONING. YOU HAVE HEALED YOUR DIASTASIS RECTI.

Now what?

Healing diastasis recti is not a destination. It is a skill you maintain for the rest of your life — through subsequent pregnancies, through menopause, through the natural aging process. Your core is not something you fix once and forget. It is something you maintain, like any other part of your health.

This chapter covers how to keep your core strong and healthy after closure, what to do if you have another baby, how aging affects your connective tissue, and how to avoid falling back into old habits.

Maintaining Core Health After Closure

The good news: once your gap is functionally closed (less than two finger-widths and no symptoms), the heavy work of healing is behind you. You do not need to be as strict about avoiding every crunch and plank. You can gradually return to a wider range of activities.

But "gradually" is the key word. A healed core is a well-maintained core.

Continue core work 2-3 times per week for life.

The deep core — TVA, pelvic floor, diaphragm, multifidus — responds to regular training. If you stop doing core work entirely, these muscles will gradually weaken. Your TVA will become less responsive. Your pelvic floor will lose tone. Your posture will start to slip. And over time, the gap may widen again — not to its original state, but enough to cause symptoms.

Two to three sessions per week of 15-20 minutes is enough to maintain what you have built. You can follow the exercises in your core program, or you can rotate in new exercises as long as they meet the safety criteria: no coning, no doming, good form, controlled breathing.

You do not need to be obsessive. A missed week is not a crisis. But if you stop entirely for months, you will lose ground.

Do a breath check on a regular basis.

Every few weeks, take a moment to check your breath. Lie on your back, place one hand on your ribs and one on your belly, and take five breaths. Are you breathing into your ribcage, or have you drifted back into belly breathing?

Belly breathing is not the enemy for a healed core — it is a normal variant of breathing. But if you find yourself belly breathing constantly during exercise and daily activities, the pressure is still pushing through your midline. A periodic breath check keeps you aware of the pattern.

Maintain awareness of form in all exercises.

When you have healed your diastasis, you can return to many exercises you avoided during treatment — including crunches, planks, and pushups. But form matters more than ever. If you do crunches with a flared ribcage and a slack core, you are still pushing pressure into your midline.

For any exercise, the same rules apply: - Ribs stacked over pelvis. - TVA engaged before you start the movement. - Exhale on effort. - If you feel coning or doming, back off.

You may find that after healing your diastasis, you prefer the deep core exercises over the traditional ab exercises. Many women do. The deep core work feels more functional. Crunches start to feel superficial and unsatisfying in comparison.

Stay on top of your pelvic floor.

The pelvic floor connection does not end when your gap closes. Your pelvic floor will continue to need maintenance — especially through subsequent pregnancies, heavy lifting phases, and menopause. Continue pelvic floor awareness work. Do Kegels as needed. And if you notice any leaking or pelvic pressure, address it early.

A small leak is not a reason to panic. But it is a signal that your pelvic floor needs some attention. A few days of focused Kegel and breath work can often resolve it. If it persists, see a pelvic floor PT.

What Subsequent Pregnancy Means

If you are planning to have another baby, you may wonder: will pregnancy undo all my work?

The answer is: it depends, but the outlook is good for most women.

Many women with healed diastasis have successful pregnancies without re-separation.

If your core is well-healed — the linea alba is firm, the gap is less than two finger-widths, and you have good core function — the linea alba will stretch during pregnancy, but it is more likely to snap back after birth than it was the first time.

The stretching of the linea alba during pregnancy is normal. All pregnant women experience some separation. The question is whether the connective tissue has enough integrity to return to its pre-pregnancy state after birth. A well-healed linea alba has better collagen structure and better tension. It is more resilient.

But some recurrence happens.

If you had a wide diastasis before treatment — or if your tissues were thin and lax even after treatment — there is a higher chance of recurrence. Each subsequent pregnancy stretches the tissues further. The body does not always fully recover to the previous baseline.

Also, if you carried low with your first pregnancy or had a large baby, you may have more stretching with subsequent pregnancies regardless of your core strength.

Pregnancy-safe core work is critical.

During pregnancy, you need to continue deep core work — adapted for pregnancy and clearance from your doctor. Ribcage breathing, TVA engagement, gentle glute bridges,

and pelvic floor awareness are all safe during pregnancy as long as you have no contraindications from your healthcare provider.

Avoid any exercise that creates coning or doming during pregnancy. Your linea alba is softer under the influence of relaxin and progesterone, so it is even more vulnerable to pressure.

After birth.

Treat yourself as postpartum again, even if you have healed your diastasis before. Perform the self-check at 6-8 weeks postpartum. Start the core progression at the appropriate level. Your core has been through a structural event, and it needs recovery time.

Many women find that their postpartum recovery is faster the second time because they know what to do. They have the skills. They can start working on their core immediately. That is an advantage.

Aging with a Healed Core

Your core will change as you age, just as every other part of your body changes. But the work you do now will pay dividends for decades.

Menopause and connective tissue health.

The hormonal changes of menopause affect connective tissue. Estrogen plays a role in collagen synthesis and maintenance. As estrogen levels decline during menopause,

collagen production decreases. The skin, the tendons, the ligaments — and the linea alba — all become less elastic and more prone to injury.

This does not mean your diastasis will automatically return at menopause. But it does mean that your connective tissue will need different support than it did in your thirties and forties.

The role of collagen.

As you age, your body produces less collagen. The collagen it does produce is of lower quality. Two strategies can help:

1. **Dietary support.** Adequate protein intake is essential for collagen synthesis. So are vitamin C and zinc. As you age, make sure your diet includes enough of these nutrients. Collagen peptide supplements may be helpful, but whole food sources of protein and collagen-supporting nutrients are the foundation.
2. **Mechanical loading.** Your body produces more collagen in tissues that are mechanically loaded. Resistance training — including core work — signals your body to maintain the connective tissue in those areas. A sedentary lifestyle accelerates the decline. Active lifestyle maintains it.

Resistance training for connective tissue health.

Resistance training — lifting weights, bodyweight exercises, using resistance bands — is one of the best things you can do for connective tissue health as you age. Each time you load a muscle, you also load the associated connective tissue. The tissue responds by remodeling and strengthening.

You do not need to be a heavy lifter. Moderate, consistent resistance training — two to three times per week — is sufficient to signal your body to maintain its connective tissue.

The exercises are a good foundation. As you progress, you can add squats, lunges, rows, presses, and deadlifts — all with proper form and core engagement.

Paying attention to changes.

As you age, pay attention to changes in your core. If you notice new coning or doming, increased back pain, or changes in your pelvic floor function, do a self-check. Your gap may be widening slightly, and early intervention is easier than waiting until it is a significant problem.

A return of symptoms does not mean you have failed. It means your body has changed, and you need to adjust your maintenance.

Staying Out of the Old Habit Traps

The most common reason for recurrence of diastasis symptoms is not aging or pregnancy. It is falling back into old movement habits.

The crunch trap.

You take a class. The class does crunches. You have been doing core work for years, you feel like your core is fine, so you join in. Nothing happens the first time. Or the second time. But over months of doing crunches regularly, the gap starts to widen. By the time you notice, you have lost ground.

The trap is that the consequences of poor form are delayed. You do not feel the gap widening in real time. By the time you notice, it has been happening for weeks or months.

Prevention: stay consistent with your maintenance core work, and stay aware of your form. If you do crunches, do them with TVA engagement and ribcage breathing — and check for coning. If you see coning, stop.

The slouch trap.

You are tired. You have been sitting in a chair for hours. Your ribs collapse, your pelvis tucks, and you slouch. It feels comfortable. But that slouched position is holding your rectus abdominis in a shortened, slack state and reducing the tension on your linea alba.

Posture check: set a timer on your phone. Every hour, do a quick body scan: - Are my ribs stacked over my pelvis? - Is my pelvis neutral or tucked? - Are my shoulders rounded or relaxed? - Am I breathing into my ribcage or my belly?

This micro-habit — five seconds every hour — will keep you from drifting into the postural patterns that weaken your core.

The "I'm fine, I don't need to do core work anymore" trap.

This is the most insidious trap of all. You have healed your diastasis. You feel great. Your core works. You stop doing the maintenance work. Weeks turn into months. Then you lift something heavy, feel a twinge, and realize you have lost some of your core stability.

The beautiful thing about the core is that it responds quickly. A few weeks of consistent work can bring back what you lost. But prevention is easier than recovery. Make core maintenance a permanent part of your routine, the way you brush your teeth or drink water.

Living Full

The goal of healing your diastasis recti is not to spend the rest of your life thinking about your core. It is to heal your core so you can stop thinking about it and get on with living.

A healed core means you can: - Lift your children without pain or coning. - Run, jump, and play without leaking. - Carry groceries in from the car without bracing beforehand. - Get out of bed without thinking about your technique. - Exercise with confidence, knowing your core can handle the load. - Feel strong and capable in your body.

That is the goal. A core that works. A body you can trust. A life you can live fully, without being limited by a separated midline.

Your core is not broken anymore. It is healed. And now you get to use it.

Toolkit Summary

- **Maintain core work 2-3 times per week for life**
 - do a periodic breath check and maintain awareness of form in all exercises. Your core is not a one-time fix.

- **Subsequent pregnancy is possible for most women with healed diastasis** — but recurrence can happen. Use pregnancy-safe core work, and treat yourself as postpartum again after birth, with a self-check at 6-8 weeks.
- **Aging affects connective tissue health through hormonal changes and reduced collagen production** — maintain core work (especially resistance training) and dietary support (protein, vitamin C) to support connective tissue as you age.
- **Avoid the habit traps** — falling back into crunches with poor form, slouching through the day, and stopping core maintenance entirely. Prevention is easier than recovery.
- **The goal is a core you do not have to think about** — a functional, pain-free core that lets you live fully. You have done the work. Now go live with it.

resources

THIS SECTION BRINGS TOGETHER THE MOST USEFUL REFERENCE MATERIAL FROM ACROSS THE BOOK IN ONE PLACE. USE IT AS A QUICK GUIDE WHEN YOU NEED TO CHECK YOUR TECHNIQUE, TRACK YOUR PROGRESS, OR REMEMBER WHAT TO SWAP FOR WHAT.

Glossary

Abdominoplasty. A surgical procedure that removes excess skin and tightens the abdominal wall. When combined with diastasis recti repair, it includes plication of the linea alba to close the muscle gap.

Coning. A visible ridge that appears along the midline of the abdomen when intra-abdominal pressure pushes against the weakened linea alba. Often seen during crunches, sit-ups, or getting up from lying down.

Core canister. The model of the core as a cylinder with four walls: the diaphragm (top), the pelvic floor (bottom), the transverse abdominis (front and sides), and the multifidus and deep spinal muscles (back). All four must work together for optimal core function.

Diaphragmatic breathing. Deep breathing that engages the diaphragm. For diastasis recti, the preferred form is ribcage-dominant (expanding the lower ribs sideways and backward), as opposed to belly breathing (expanding the abdomen forward).

Diastasis recti. A separation of the two sides of the rectus abdominis muscle along the connective tissue midline (the linea alba). Common after pregnancy but can also result from abdominal surgery, heavy lifting, obesity, and connective tissue disorders.

Doming. A localized bulge at the midline of the abdomen, often visible during core engagement or movement. Similar to coning but appears as a round protrusion rather than a ridge.

Fascial. Relating to fascia, the web-like connective tissue that wraps around muscles, organs, and bones. Fascial connections throughout the abdomen are affected by the stretching of pregnancy and play a role in core function.

Intra-abdominal pressure (IAP). The pressure inside the abdominal cavity. It is created by the diaphragm pushing down and the abdominal muscles and pelvic floor containing that force. Proper IAP management is essential for core stability and diastasis healing.

Kegel. An exercise that contracts the pelvic floor muscles. Named after Dr. Arnold Kegel. The proper technique involves a gentle lift-and-hold of the pelvic floor without holding the breath or engaging the glutes and thighs. The "elevator" cue — lift, hold, and fully release — helps avoid common form mistakes.

Linea alba. The band of connective tissue running vertically from the sternum to the pubic bone, separating the two sides of the rectus abdominis. It is made primarily of

collagen and transmits force across the midline. Stretching or thinning of the linea alba is the defining feature of diastasis recti.

Neutral spine. The natural alignment of the spine when it is in its strongest, most stable position — not excessively arched (anterior tilt) or flattened (posterior tilt). Finding neutral spine is important for core exercises and daily posture.

Pelvic floor. The sling of muscles at the bottom of the pelvis that supports the bladder, uterus, and rectum. It works in coordination with the diaphragm and deep abdominal muscles to manage intra-abdominal pressure.

Plication. A surgical technique in which the surgeon sutures (stitches) the linea alba to bring the rectus muscles back together. This is the core of abdominoplasty with diastasis repair.

Prolapse. The descent of a pelvic organ (bladder, uterus, or rectum) into or out of the vaginal canal. Often associated with pelvic floor weakness and can be linked to diastasis recti.

Rectus abdominis. The pair of vertical muscles running from the ribcage to the pubic bone, commonly known as the "six-pack" muscles. In diastasis recti, these muscles separate along the midline.

Transverse abdominis (TVA). The deepest layer of the abdominal muscles, wrapping around the torso like a corset. The TVA is the primary muscle responsible for stabilizing the core and closing the diastasis gap. Often called the "deep core" muscle.

Self-Check Instructions and Tracking Template

Self-Check Instructions

1. Lie on your back on a firm, flat surface. Bend your knees and place your feet flat on the floor, hip-width apart. Arms at your sides.
2. Place two or three fingers on the midline of your abdomen, just above your belly button, pointing toward your feet.
3. Gently lift your head and shoulders off the floor — a small curl, just enough to engage the rectus muscles. Do not do a full sit-up.
4. With your head and shoulders lifted, probe the midline with your fingers. Feel for the gap between the rectus muscles. Count how many finger-widths fit in the gap.
5. Measure at three points: at the belly button, 2 inches (approximately 2 finger-widths) above the belly button, and 2 inches below the belly button.
6. Note the depth of the gap (how far your fingers sink) and the quality of the connective tissue (firm and springy, moderate, or thin and squishy).
7. Check for coning or doming — look for a ridge or bulge at the midline during the lift.
8. Record your results in the tracking template below.

Monthly Tracking Template

Date	Gap at Belly Button (finger- widths)	Gap 2 Inches Above (finger- widths)	Gap 2 Inches Below (finger- widths)	Depth (shallow/ moderate/ deep)	Tissue Quality (firm/ moderate/ thin)	Coning/ Doming (yes/ no)	Notes

Exercise Tracking Template

Week	Dates	Daily Practice (min)	Full Workouts (#)	TVA Engagement Quality	Notes
1					
2					
3					
4					
(continue monthly)					

Safe Exercise Progression Flowchart

Start here → **Level 1: Breathing and TVA in Supine** - Ribcage breathing (1 min) - Exhale engagement (1-2 min) - TVA holds (2-3 min) - Daily practice: 5-10 min

↓ When you can maintain TVA engagement through breath cycles without coning

Level 2: Heel Slides - 8-12 reps per side, 2-3 sets - Maintain TVA engagement, no back arching

↓ When you can slide to full extension without back arching or coning

Level 3: Marching - 8-12 reps per side, 2-3 sets - Lift foot 1 inch off floor, keep knee bent

↓ When you can lift foot without back arching or coning

Level 4: Dead Bug - 6-10 reps per side, 2-3 sets - Extend arm and opposite leg, keep foot/hand just above floor

↓ When you can extend fully without back arching or coning

Level 5: Side-Lying Work - Clamshells: 10-15 reps per side, 2-3 sets - Side-lying leg lifts: 8-12 reps per side, 2-3 sets

↓ When you can complete reps with good form

Level 6: Glute Bridge - 10-15 reps, 2-3 sets - Press through heels, squeeze glutes at top

↓ When you can bridge without back arching or coning

Level 7: Bird Dog (Knees-Down Option) - 6-10 reps per side, 2-3 sets - Extend arm and opposite leg, keep torso still

↓ When you can extend fully without hip rotation, back arching, or coning

Level 8: Standing Core and Functional Movement - Standing heel drops - Squat with core engagement - Walking lunges - Single-leg balance

↓ Gradual integration into full fitness routine

Exercise Substitution Cheat Sheet

Instead of This	Do This	Why It Works
Crunches	Dead bugs	Same rectus activation, less midline pressure
Sit-ups (full range)	Heel slides	Core stabilization without spinal flexion
Planks (on hands or forearms)	Bird dog with knees down	Stabilizer work without sustained high pressure
Full pushups (on floor)	Incline pushups (counter or wall)	Pressing strength with less midline stress
Supine leg raises	Marching (on back, feet on floor)	Challenges hip flexor and core without flexion
Double leg lowers	Heel slides or single leg extensions	Same pattern with less load
Russian twists	Side-lying clamshells	Oblique and lateral core work safely
Bicycle crunches	Bird dog with arm/leg extension	Rotational stability without twisting
Burpees	Squat + standing heel drops	Power and conditioning without midline stress
Toe touches	Dead bugs	Leg-over-head pattern with appropriate loading
Ab crunch machines	TVA engagement holds	Real deep core work, not superficial

Instead of This	Do This	Why It Works
V-ups / Jackknives	Dead bugs on unstable surface	Same intensity, safe pattern
Roll-ups (Pilates)	Pelvic tilts + cat- cow	Spinal articulation without flexion pressure
Boat pose (yoga)	Glute bridge	Core stability without sustained flexion
Hundred (Pilates)	Ribcage breathing + TVA holds	Breath and core coordination, no harmful pressure

Postpartum Movement Guide

Lifting Your Baby

- **Squat, don't bend.** Bend at your knees and hips, not your waist. Keep your chest up.
- **Exhale on the lift.** Inhale before lifting, exhale as you stand. Never hold your breath.
- **Keep baby close to center.** Bring your baby against your chest before standing.
- **Brace your core before every lift.** Gently engage your TVA before you start the movement.

Carrying Your Baby

- **Center your baby on your chest** in a structured carrier with a supportive waistband.

- **Avoid carrying on one hip for long periods.** If you must hip-carry, alternate sides.
- **Keep your ribs stacked over your pelvis** when wearing your baby — do not lean backward to compensate for the weight.
- **As your baby gets heavier, use a structured carrier** that transfers weight to your hips.

Getting Out of Bed (Log Roll)

1. Bend your knees, feet flat on the bed.
2. Roll your entire body onto your side — like a log, no twisting.
3. Use your top arm to push yourself up to seated.
4. Let your legs drop off the side as you push up.
5. **STAND USING YOUR LEGS, NOT YOUR BACK.**
6. **DO NOT SIT UP WITH STRAIGHT LEGS.**
7. **Do not lead with your head.** Keep your neck long.

Car Seats, Groceries, and Laundry

- **Use two hands for any heavy or awkward load.**
- **Squat to lift from the floor or from a car.** Bring the load close to your body.
- **Divide heavy loads into smaller trips.**
- **Keep loads centered** — two balanced grocery bags are better than one heavy bag.
- **Exhale and brace before every lift.**

Nursing Station Setup

- **Sit in a supportive chair** — your hips slightly higher than your knees.
 - **Bring baby to you** — use a nursing pillow or regular pillow to prop baby to the right height.
 - **Keep ribs stacked over pelvis** — do not hunch forward.
 - **Feet flat on the floor or on a footrest.**
 - **Use feeding time for ribcage breathing practice** — place one hand on your lower ribs and one on your belly.
-

Further Reading

Books

Diastasis Recti: The Whole-Body Guide to Healing and Recovery by Katy Bowman, MS. A biomechanical approach to diastasis recti that explores how the entire body — not just the abdomen — is affected by separation and how whole-body alignment supports healing.

Heal Your Core: A Postpartum Program for Diastasis Recti by Amanda Fifer. A practical, exercise-focused program for healing diastasis recti after pregnancy, with detailed instructions and illustrations for safe core exercises.

The Core: Understanding the Relationship Between Your Pelvic Floor, Abdominals, and Back by Dr. Sarah Duvall, PT, DPT. A physical therapist's comprehensive guide to the interconnected core system, including diastasis recti, pelvic floor dysfunction, and back pain.

Return to Light: A Mother's Journey Through Postpartum Recovery by Dr. Sarah Duvall, PT, DPT. A guided program for the early postpartum period, covering breathing, core engagement, and the pelvic floor.

The Dia Method: The 7-Day Core Restore for Postnatal Women by Leah F. A concise program focused on the breath-core-pelvic floor connection.

Strong as a Mother: A Guide to Pregnancy, Birth, and Postpartum Fitness by Kate Rowe-Ham. A comprehensive fitness guide for pregnancy and postpartum that includes diastasis-safe exercise principles.

Websites and Online Resources

The V-Hive (thehive.com). An online platform dedicated to women's pelvic health. Offers courses, articles, and videos on diastasis recti, pelvic floor health, and core recovery. A comprehensive resource for evidence-based information.

Expecting and Empowered (expectingandempowered.com). A workout app and resource for pregnancy and postpartum fitness, including diastasis-safe core exercises and pelvic floor work. Founded by a pelvic floor physical therapist.

Pelvic Guru (pelvicguru.com). A directory of pelvic floor physical therapists worldwide, plus articles and resources on pelvic health, diastasis recti, and postpartum recovery.

The Pelvic Floor Project (thepelvicfloorproject.com). A nonprofit organization providing education and resources on pelvic floor health, including diastasis recti information and practitioner directories.

Mommastrong (mommastrong.com). A postpartum fitness program founded by a pelvic floor PT that includes diastasis-safe core progressions and breathing techniques.

Instagram accounts worth following: - @expectingandempowered — Evidence-based postpartum fitness and core recovery - @pelvicguru — Pelvic floor and diastasis recti education - @thehive — Pelvic health content and community - @dr.sarahduvall — Physical therapist specializing in core and pelvic health - @mama_physio — Pelvic floor PT sharing postpartum core recovery content - @thebellemethod — Diastasis-safe core and pelvic floor exercise content

Organizations

American Physical Therapy Association — Section on Women's Health (apta.org/womenshealth). Professional organization for women's health physical therapists. Their directory can help you find a pelvic floor PT in your area.

International Continence Society (ics.org). Professional organization focused on continence and pelvic floor health. Their patient resources section offers reliable information on incontinence and pelvic floor dysfunction.

Note on sources: I am not a doctor, physical therapist, or personal trainer. The resources listed here are ones I have found helpful in my own healing and research. Always consult a qualified pelvic floor physical therapist or healthcare provider for personalized assessment and treatment. Evidence in this field continues to evolve, and today's best practices may be refined by future research.

About the Author

AUTHOR BIO GOES HERE

Also by AUTHOR_NAME

This is her first book.

